

BONES OF THE UPPER LIMB

Department of Anatomy

Musculoskeletal System

The Upper Limb (1)

Second Part

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Lecture Points

- **Skeletal System Parts.**
- **Structural Types of Bones**
- **JOINTS**
 - Definition.
 - Type of joints.
 - Typical synovial joint .
 - The accessory structures associated with synovial joint and major features.
 - Types of synovial joint according the shape.
 - Type of Cartilaginous joints:
- **Skelton of Musculoskeletal system.**
- **The Upper Limbs**
- ***The Bones of Upper Limb are:***
- ***Pectoral or Shoulder Girdle***
- **Clavicle**
 - Articulation of the clavicle
 - Muscle Origins and Insertions and Ligament attachments
 - Normal X-ray
 - Fractures of the Clavicle
- **Scapula.**
 - Fracture of the Scapula
 - Articulation of head of humerus with scapula
 - Humerus
 - Fractures of Surgical Neck and Mid shaft Humerus
 - Supracondylar Fractures.
 - Nerves affected in Fractures of Humerus
- **Bones of The Forearm**
 - Ulna
 - Radius
 - Normal X-ray Radiographic Appearances
 - Fractures of radius & ulna
 - fractured elbow
- **WRIST (CARPUS)**
- **Carpal Bones**
- **Metacarpus**
- **Digits (Phalanges).**

Scapula

- It is a **triangular**.
- It is **Flat bone**.
- Extends between the **2nd - 7th ribs**.

It has :

- **Two Surfaces:**

- 1) Concave Anterior (Costal):**

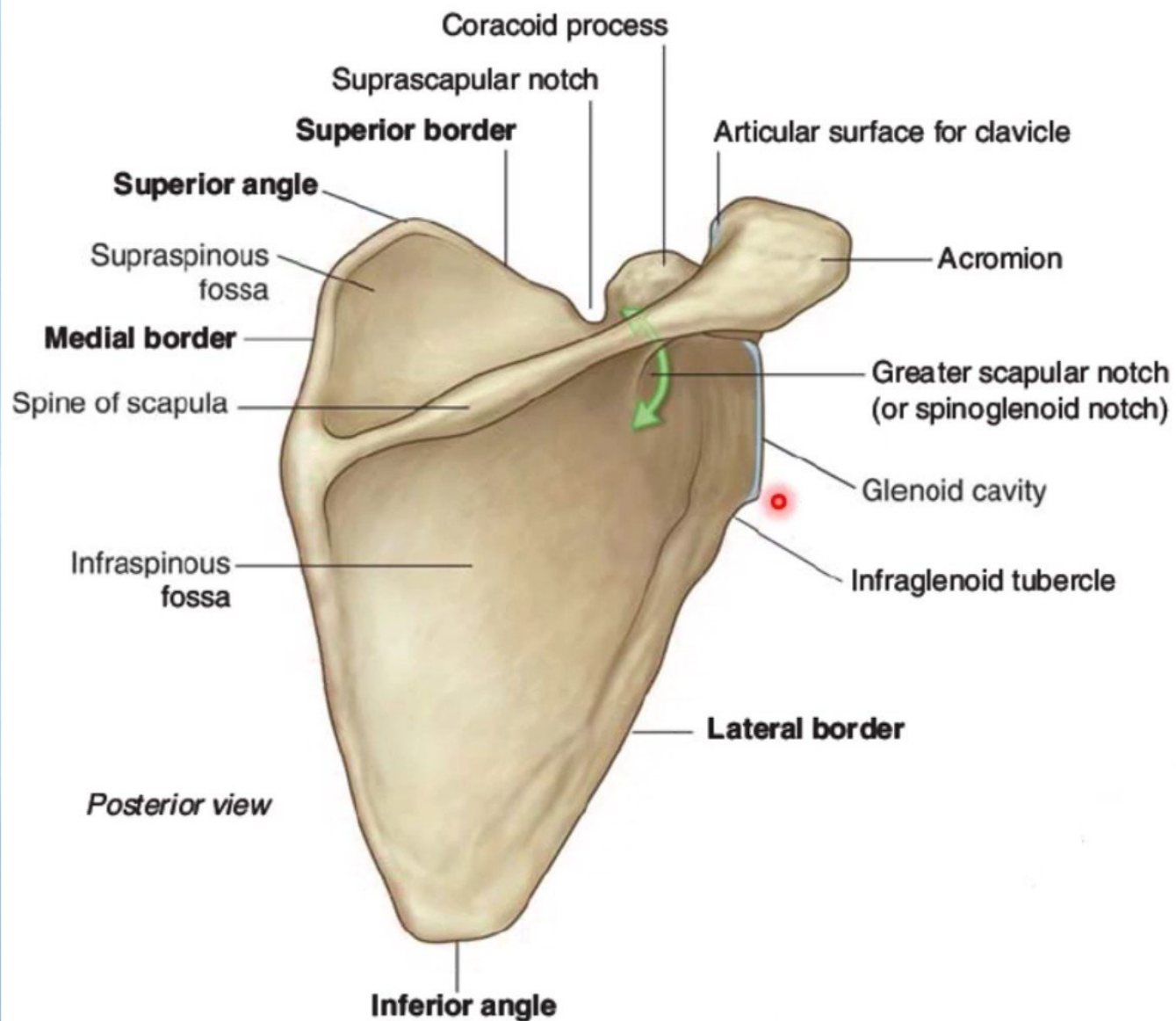
It forms the **large Subscapular Fossa**.

- 2) Convex Posterior :**

Divided by the spine of the scapula into the:

- **Smaller Supraspinous Fossa**
(above the spine).
- **larger Infraspinous Fossa**
(below the spine).

- **Suprascapular notch**



Three Angles :

- Superior
- Lateral (forms the Glenoid cavity) : a shallow concave oval fossa that receives the head of the humerus.
- Inferior

Three Processes:

(1) Spine: a thick projecting ridge of bone that continues laterally.

(2) Acromion : forms the subcutaneous point of the shoulder.

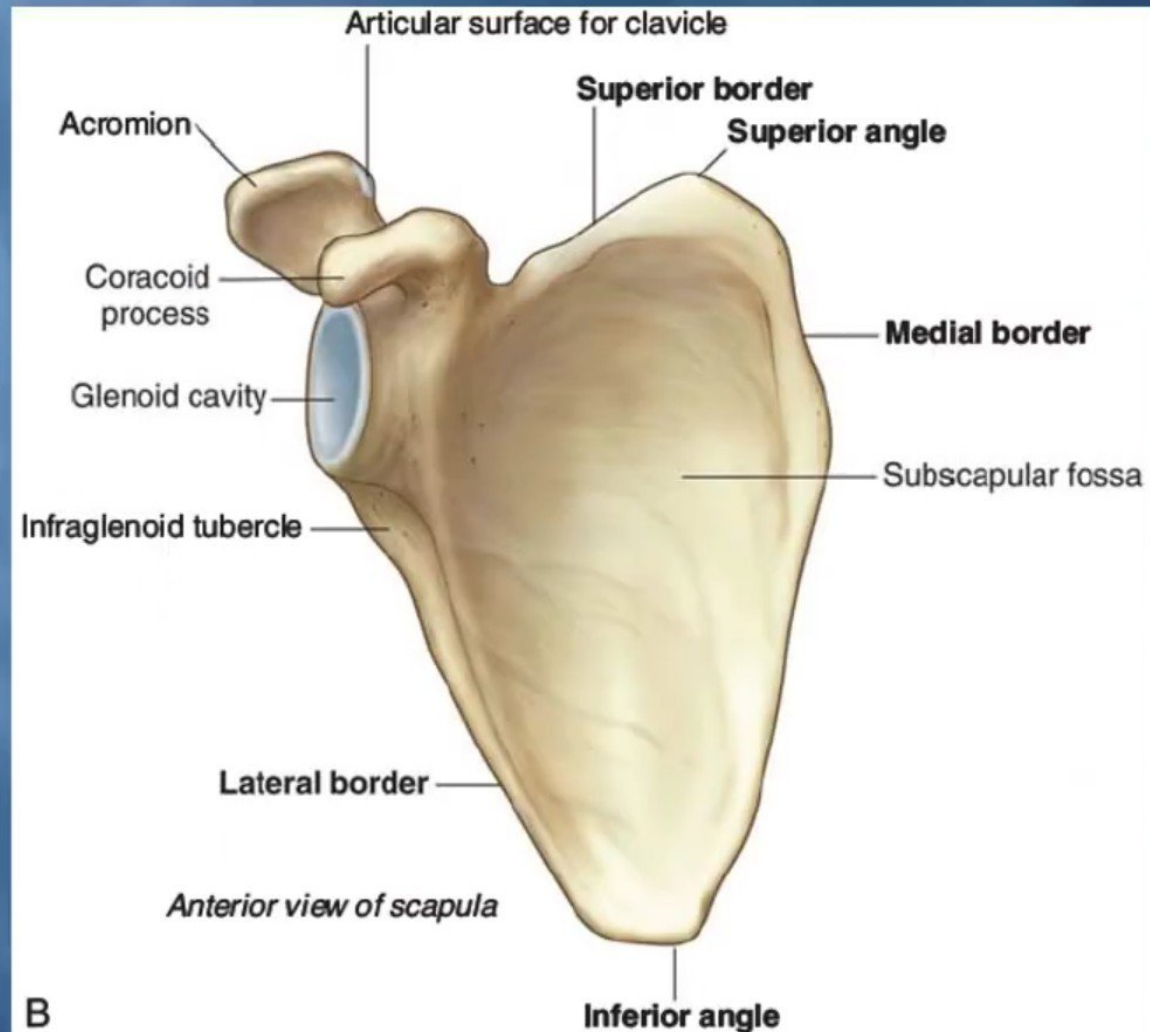
(3) Coracoid: a beak-like process.

- It resembles in size, shape and direction a bent finger pointing to the shoulder.

Three Borders:

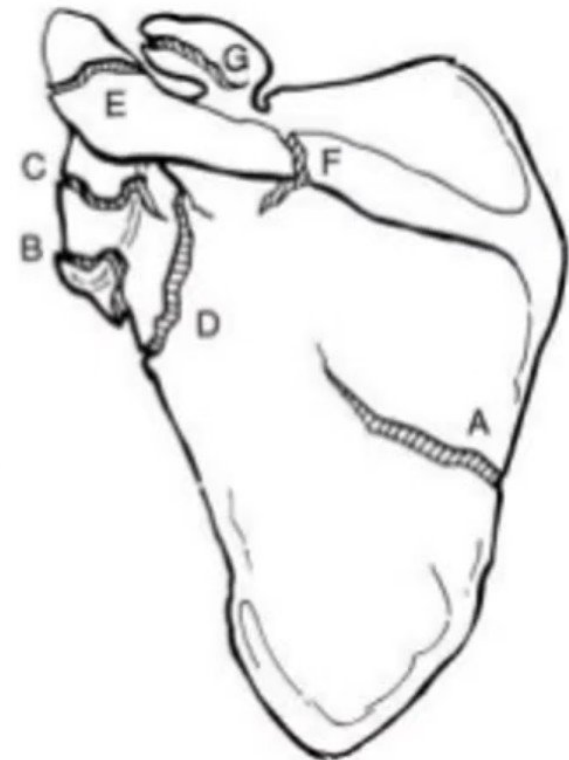
- Superior, Medial (Vertebral) & Lateral (Axillary) (the thickest) part of the bone, it terminates at the lateral angle .

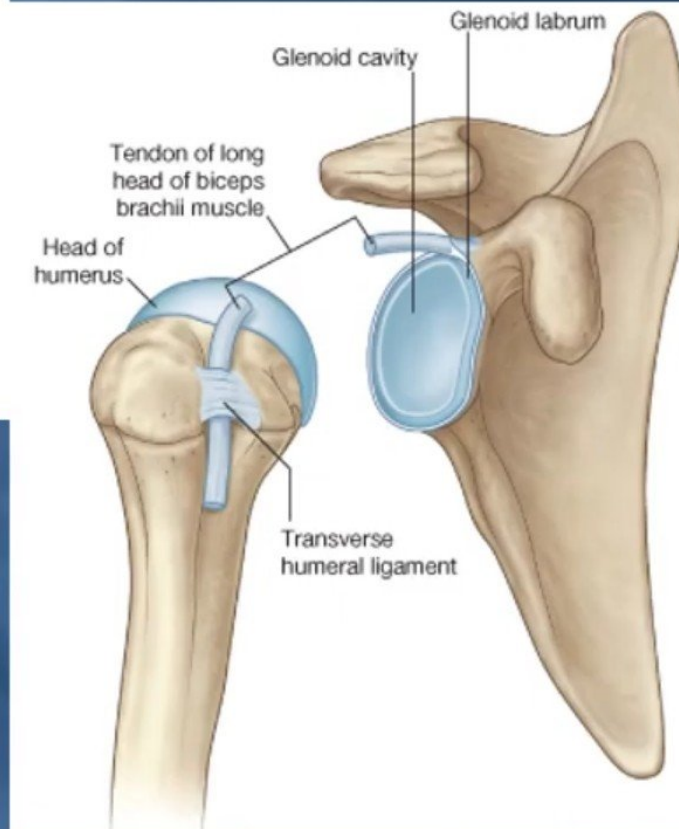
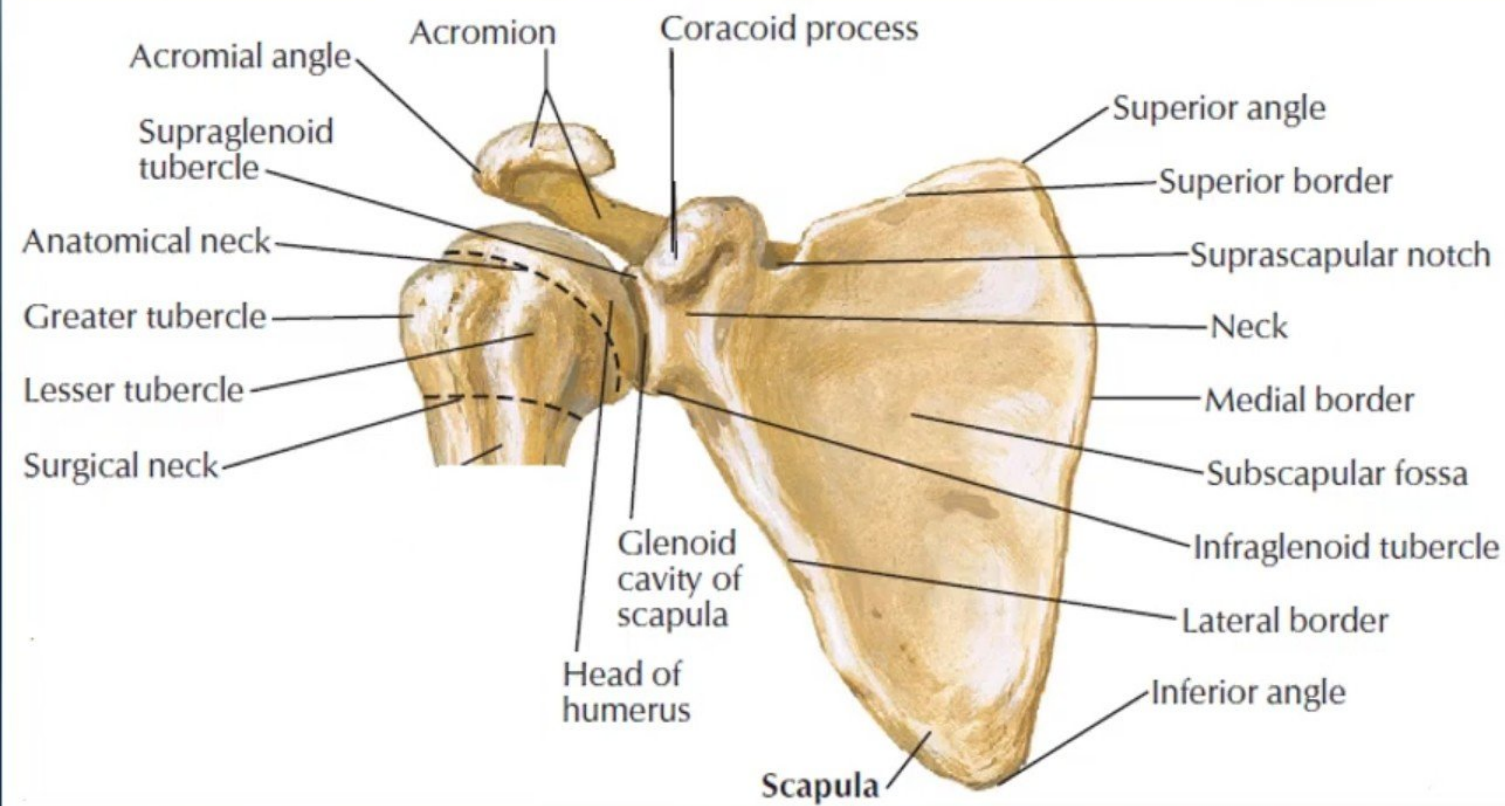
Scapula



Fracture of the Scapula

- **Definition:** Is a fracture of shoulder blade, represent an uncommon injury.
- **Types:**
 - Body (A).
 - Neck (D).
 - Type I - nonangulated, nondisplaced
 - Type IIa - shortened / displaced > 1 cm.
 - Type IIb - Angulated > 40 degree.
 - Glenoid (B C).
 - Acromian (E).
 - Coracoid (G).





Articulation of head of humerus with scapula

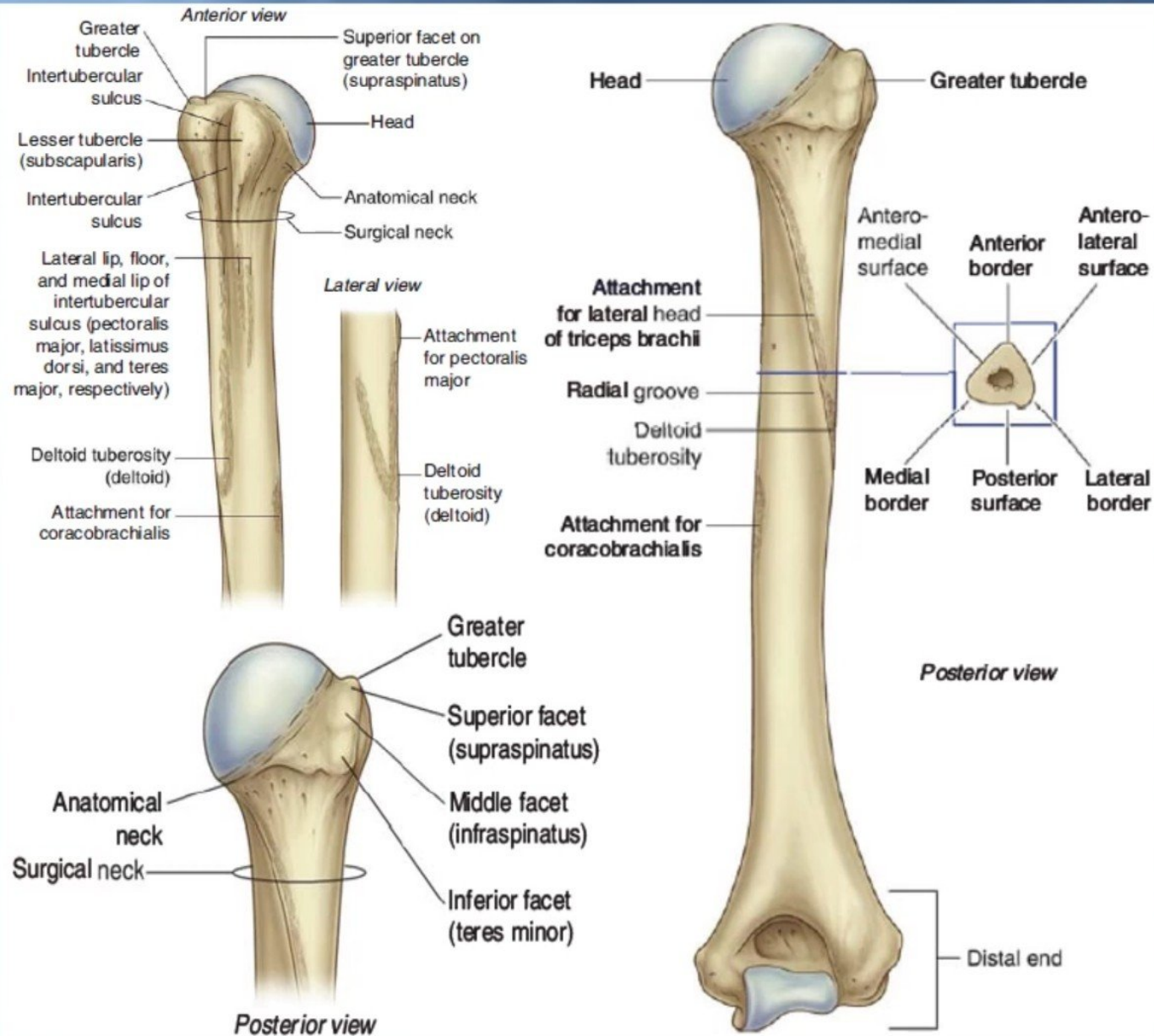
Humerus

A typical Long bone.

- It is the largest bone in the Upper Limb

Proximal End :

- Head, Neck, Greater & Lesser Tubercles.
- Head: Smooth
- It forms 1/3 of a sphere, it articulates with the glenoid cavity of the scapula.
- Greater tubercle: at the lateral margin of the humerus.
- Lesser tubercle: projects anteriorly.
- The two tubercles are separated by Intertubercular Groove.
- Anatomical neck: formed by a groove separating the head from the tubercles
- Surgical Neck: a narrow part distal to the tubercles.



Humerus

Shaft (Body):

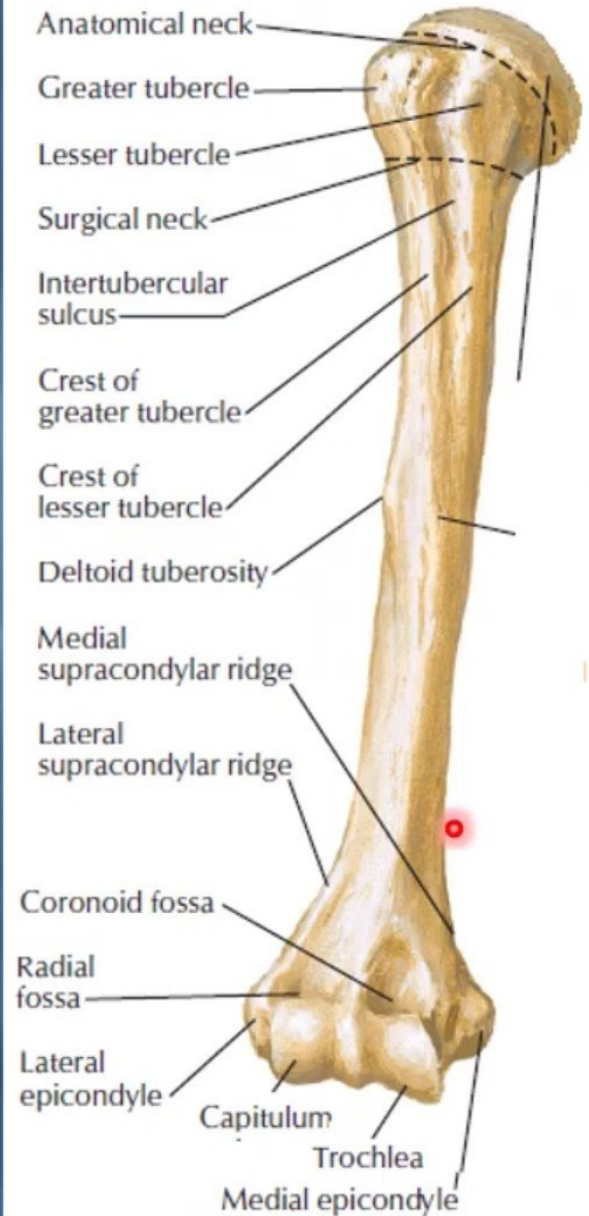
- *Has two prominent features:*

1) Deltoid tuberosity:

- *A rough elevation laterally for the attachment of deltoid muscle.*

2) Spiral (Radial) groove:

- *Runs obliquely down the posterior aspect of the shaft.*
- *It lodges the important **radial nerve** & **profunda brachii vessels**.*
- Distal End:
- *Widens as the sharp medial and lateral Supracondylar Ridges and end in the **Medial (can be felt) and Lateral Epicondyles**.*
- *They provide muscular attachment.*

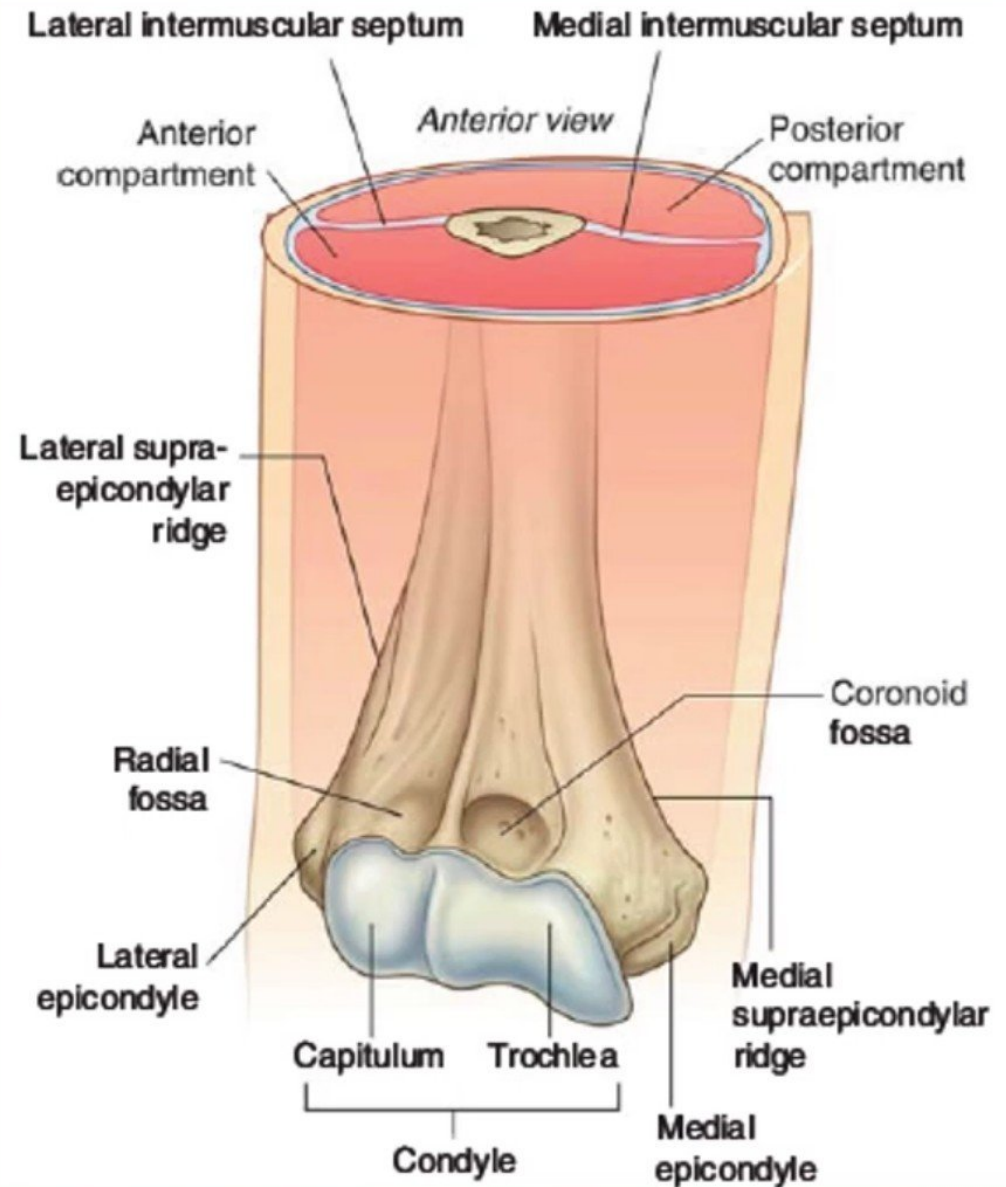


Humerus

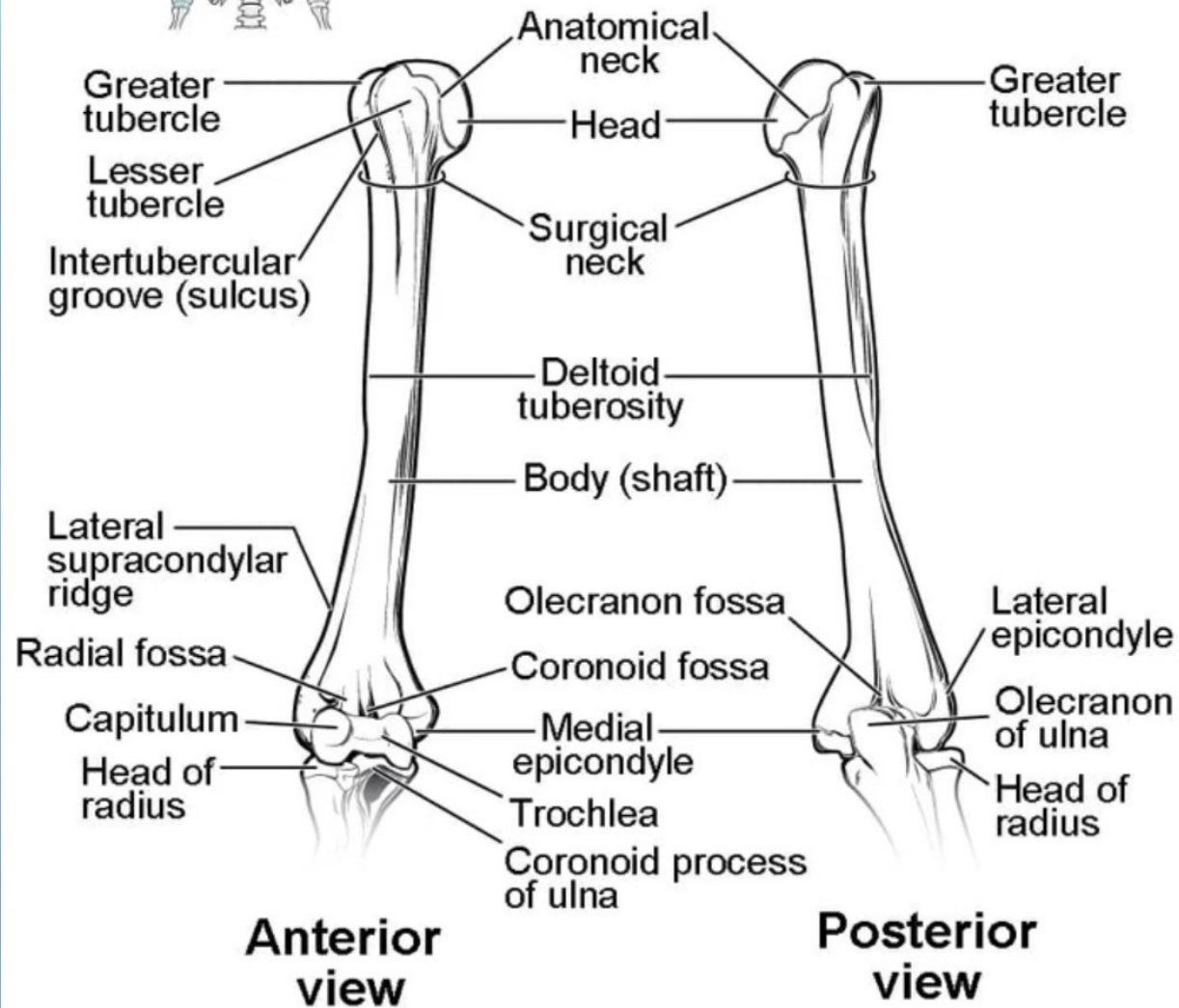


Distal end:

- Anteriorly:
- Trochlea: (medial) for articulation with the ulna
- Capitulum: (lateral) for articulation with the radius.
- Coronoid fossa :above the trochlea.
- Radial fossa: above the capitulum.
- Posteriorly:
- Olecranon fossa : above the trochlea.

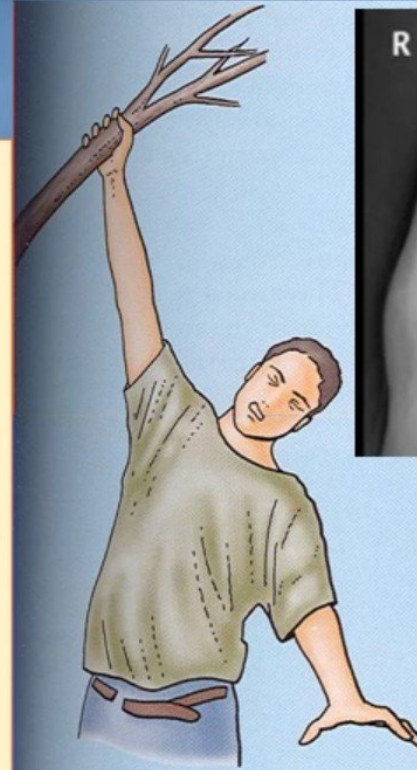


Humerus



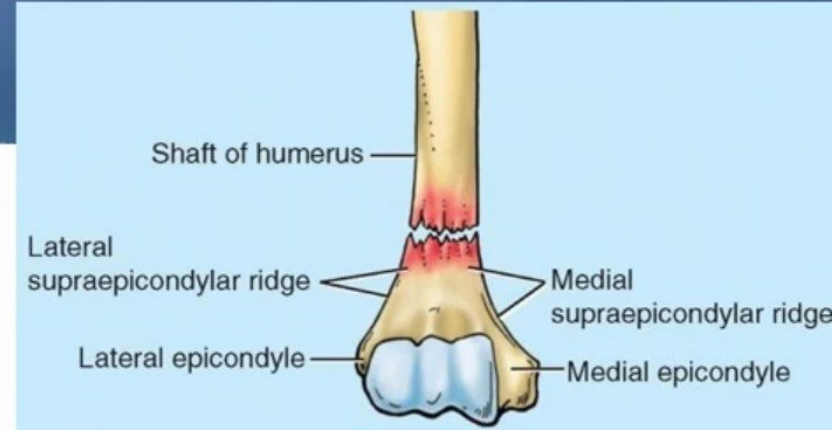
Fractures of Humerus

- Most common fractures are of the **Surgical Neck** especially in **elder people with osteoporosis**.
- The fracture results from **falling on** the hand (transition of force through the bones of forearm of the extended limb).
- In **younger people**, fractures of the **greater tubercle** results from falling on the hand when the arm is abducted .
- The **shaft of the humerus (mid shaft)** can be fractured by a direct blow to the arm or by indirect injury as falling on the outstretched hand.



Supracondylar Fractures

- A **fracture of the distal part** of the humerus, near the supraepicondylar ridges, is called a **supraepicondylar fracture**.
- The distal bone fragment may be **displaced anteriorly** or **posteriorly**.
- The actions of the **brachialis** and **triceps** tend to pull the distal fragment over the proximal fragment, **shortening the limb**. Any of the nerves or branches of the brachial vessels related to the humerus may be injured by a displaced bone fragment



Nerves Affected in Fractures of Humerus

Surgical neck:

Axillary nerve _____

Musculocuanous nerve

Not related to bone _____

Distal end of humerus

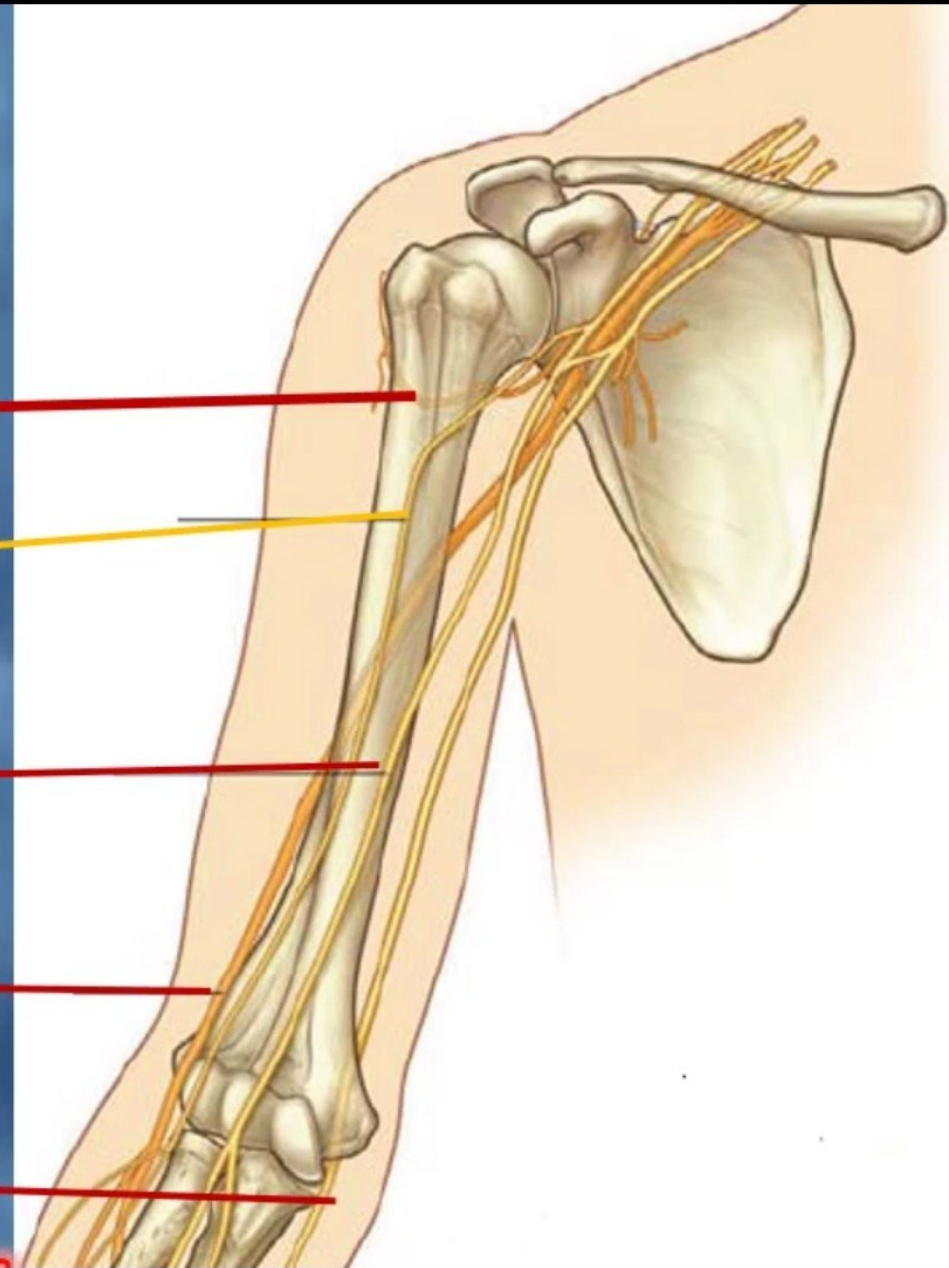
Median nerve _____

Radial groove (shaft):

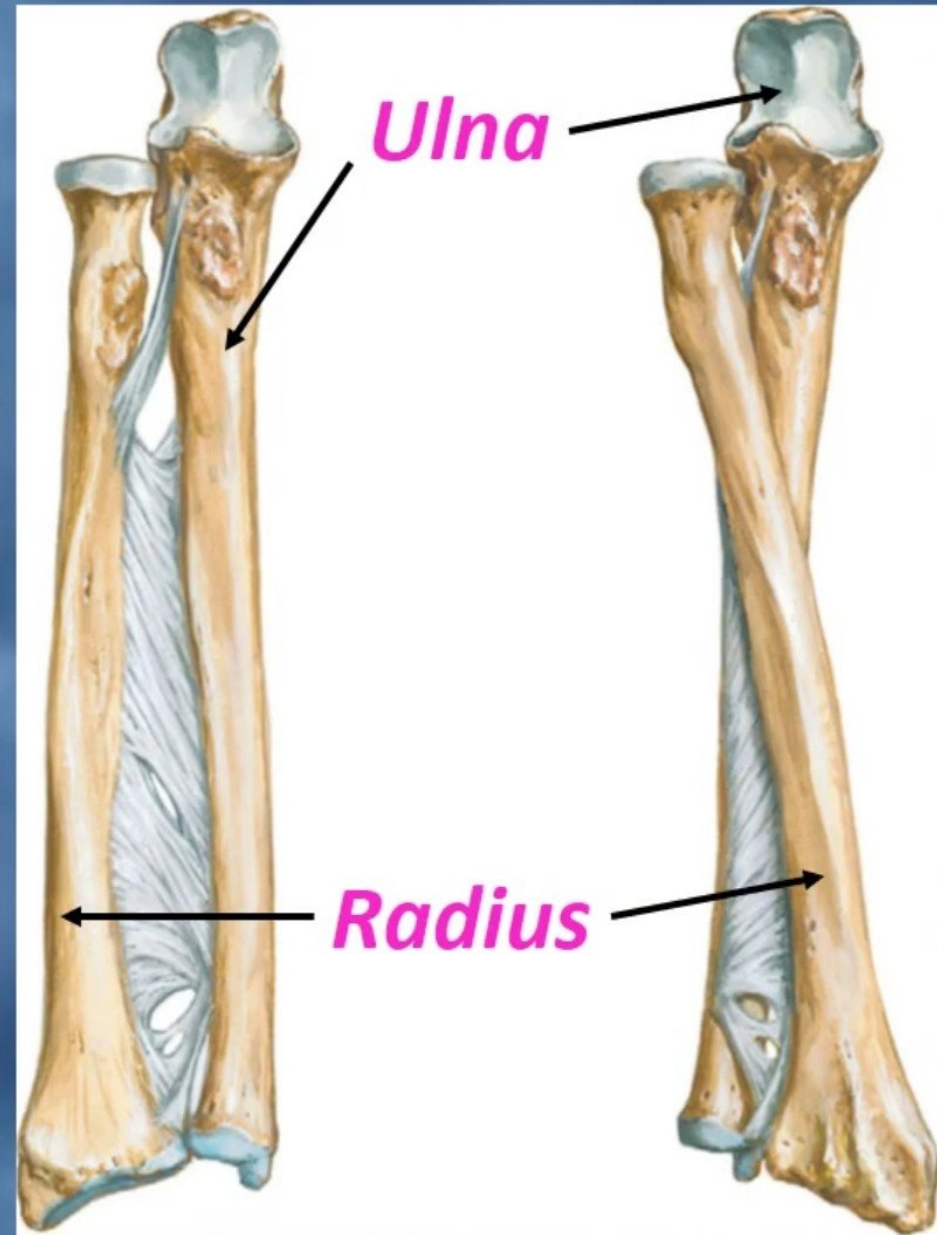
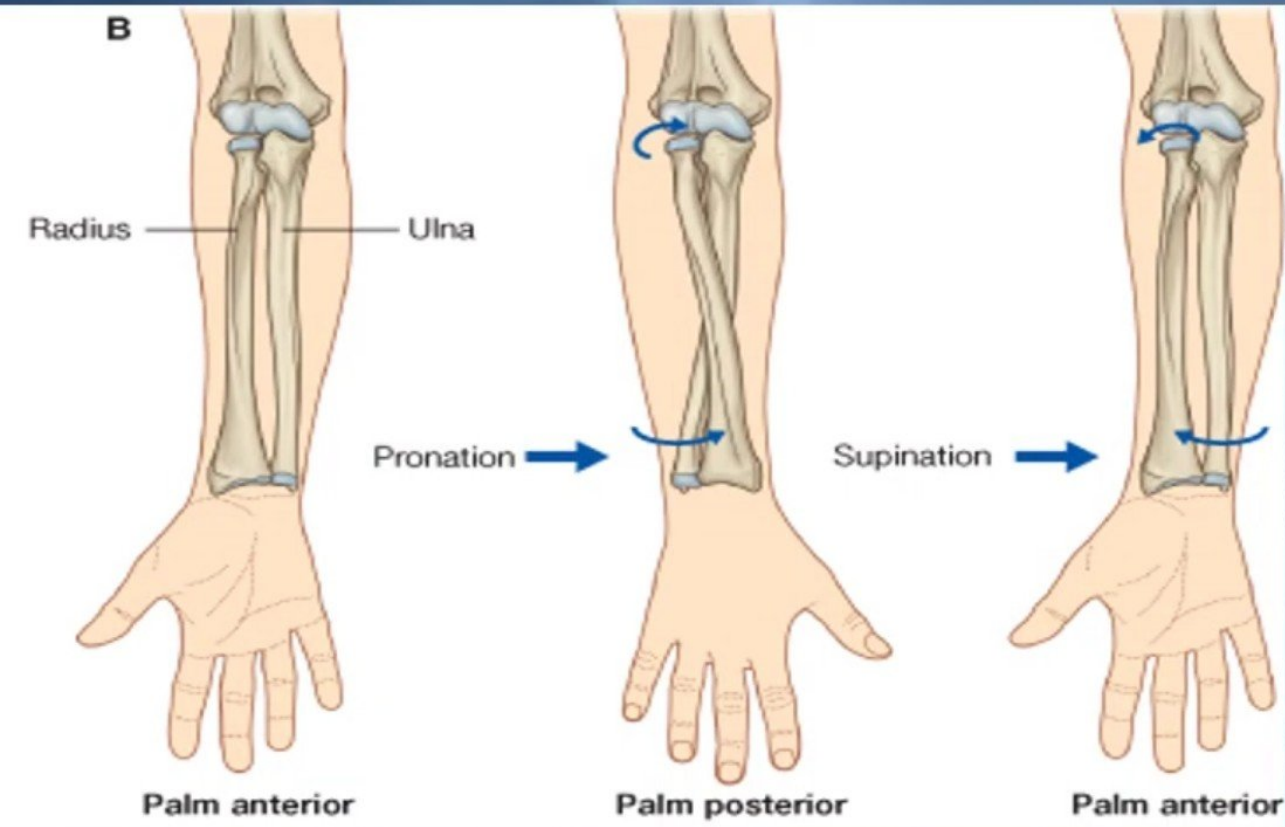
Radial nerve _____

Medial epicondyle :

Ulnar nerve _____



Bones of The Forearm



Ulna



- It is the stabilizing bone of the forearm.
- It is the medial & longer of the two bones of the forearm.

Proximal End

1. Olecranon Process :

- projects proximally from the posterior aspect (forms the prominence of the elbow).

2. Coronoid Process :

- projects anteriorly.

3. Tuberosity of Ulna:

- inferior to coronoid process.

4. Trochlear Notch:

- articulates with trochlea of humerus.

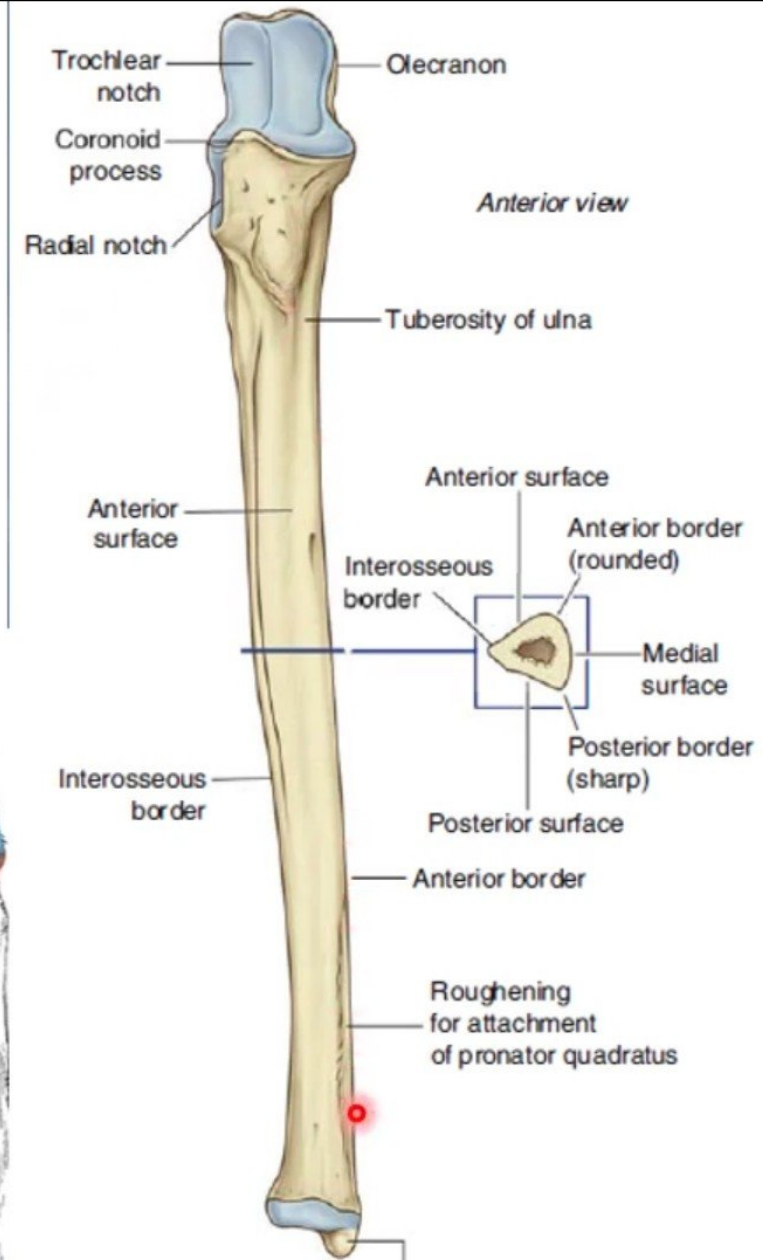
5. Radial Notch :

- a smooth rounded concavity lateral to coronoid process.

Lateral:



Anterior:



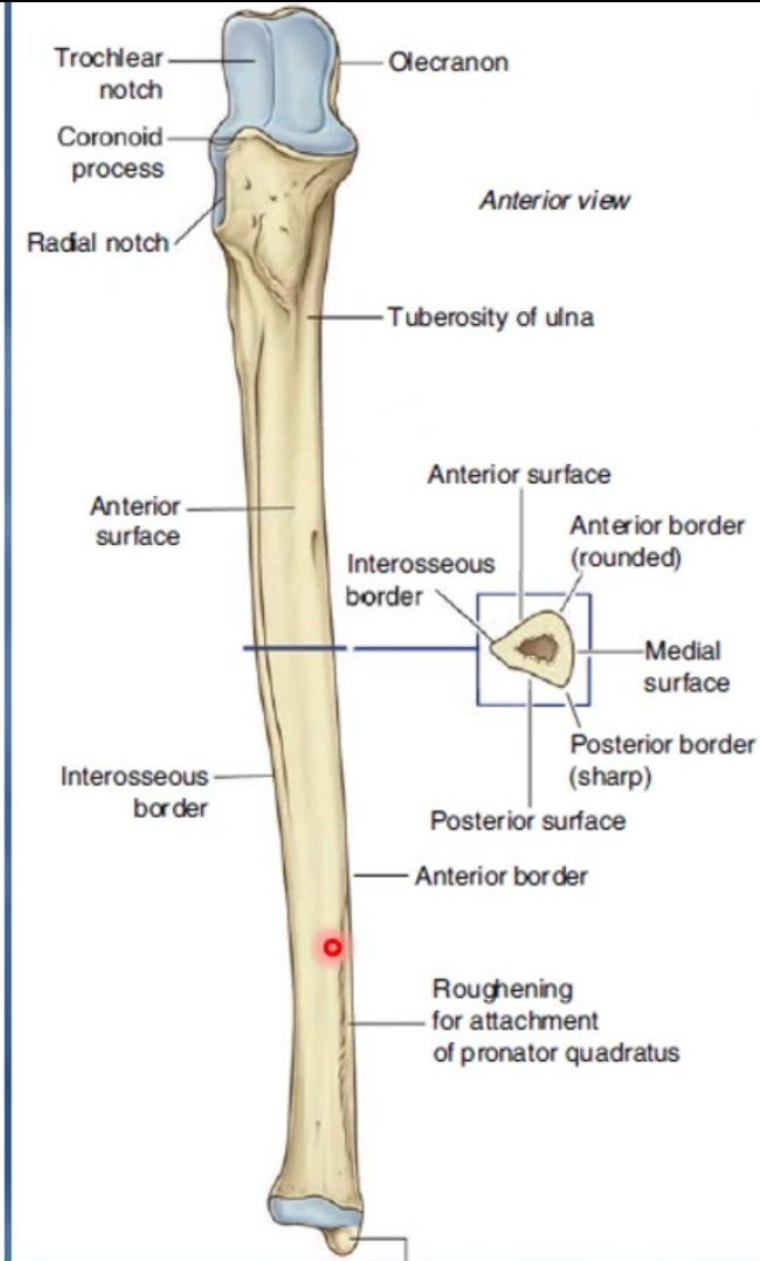
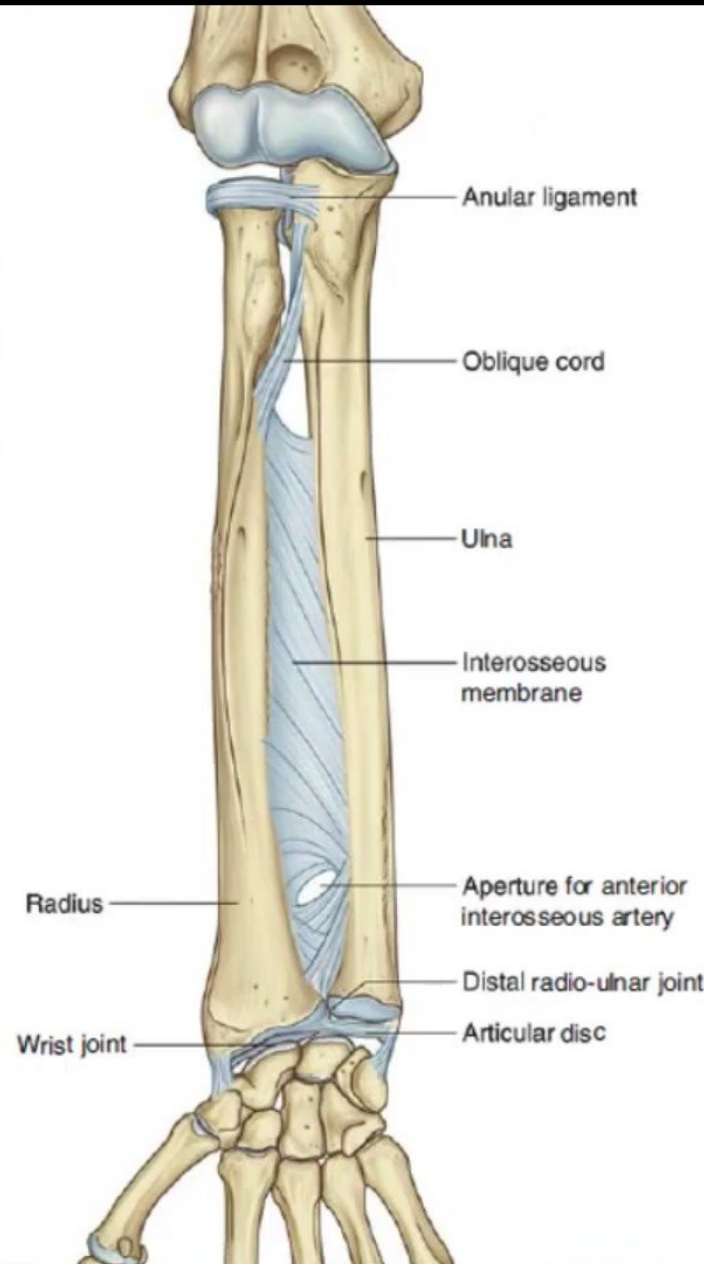
Ulna

Shaft :

- *Thick & cylindrical superiorly but diminishes in diameter inferiorly*
- *It has Three Surfaces (Anterior, Medial & Posterior).*
- *Sharp Lateral Interosseous border.*

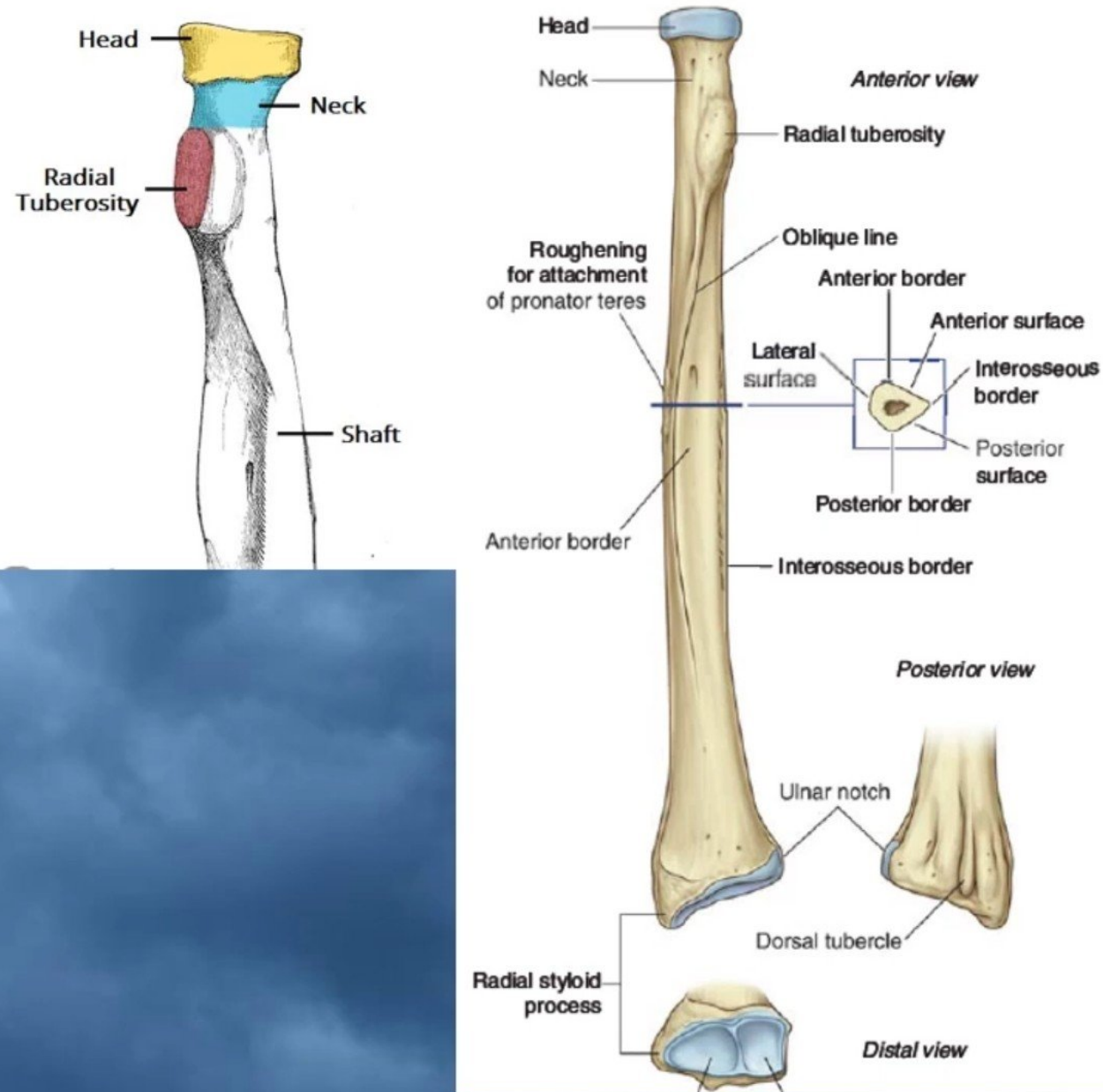
Distal End:

- *Small rounded*
1. *Head: lies distally at the wrist. .*
 2. *Styloid process: Medial.*



Radius

- *It is the shorter and lateral of the two forearm bones.*
- **Proximal End:**
- **1. Head:** small & circular
- *Its upper surface is concave for articulation with the Capitulum.*
- **2. Neck.**
- **3. Radial (Bicipital) Tuberosity :** medially directed and separates the proximal end from the body.
- **Shaft:**
- *Has a lateral convexity.*
- *It gradually enlarges as it passes distally.*



Radius

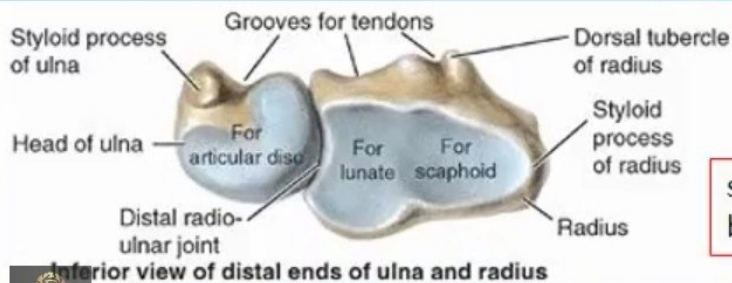
Distal (Lower) End:

It is rectangular

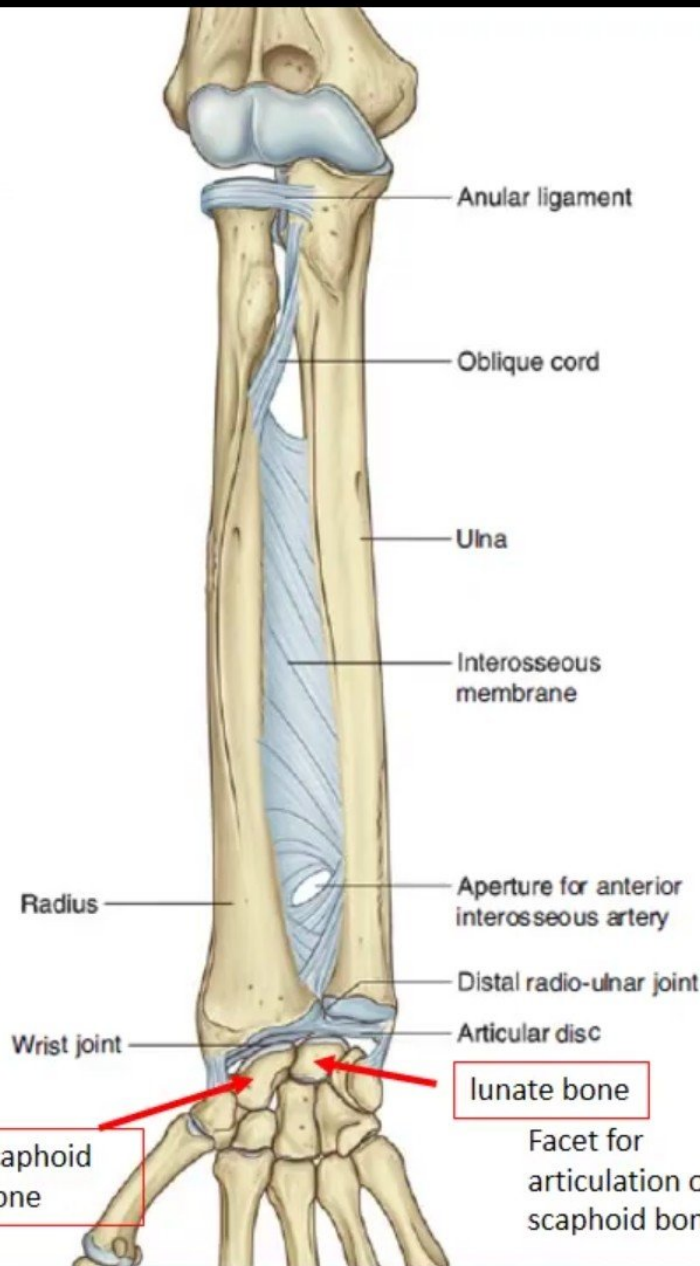
1. Ulnar Notch : a medial concavity to accommodate the head of the ulna.

2. Radial Styloid process: extends from the lateral aspect.

3. Dorsal tubercle: projects dorsally.



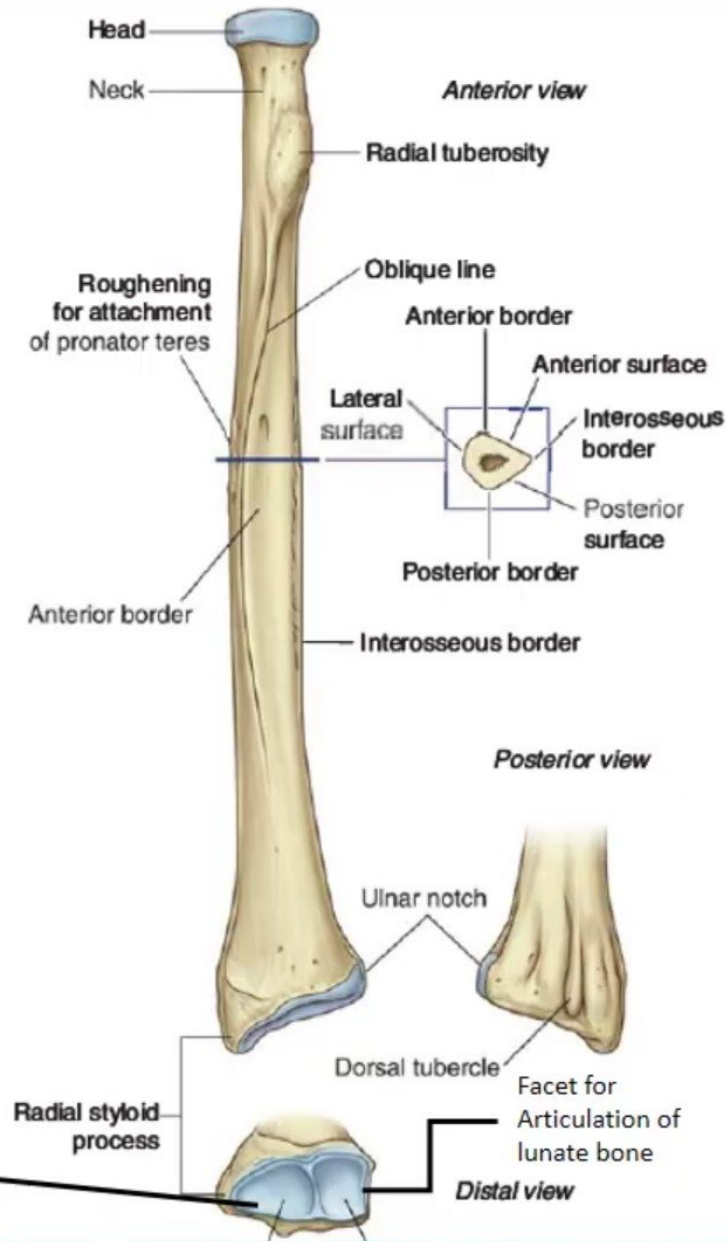
Inferior view of distal ends of ulna and radius



scaphoid bone

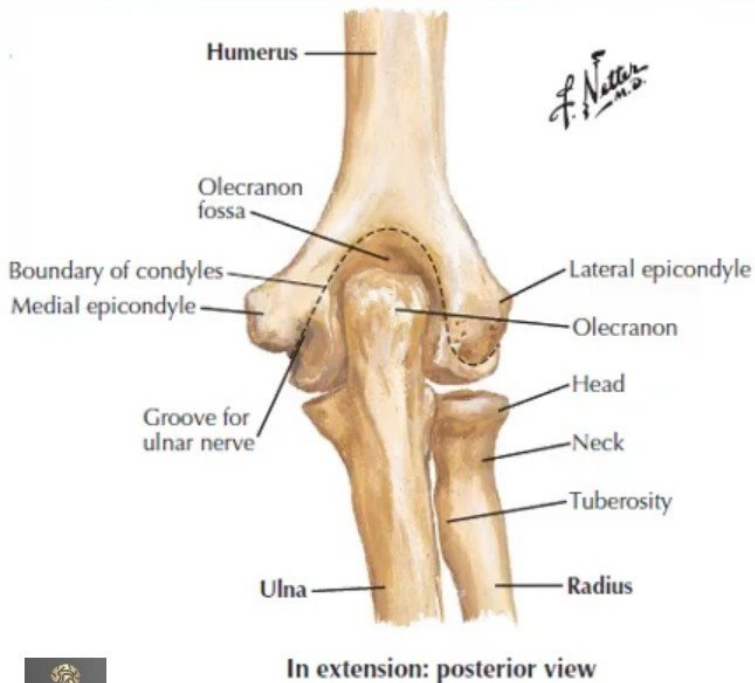
lunate bone

Facet for articulation of scaphoid bone

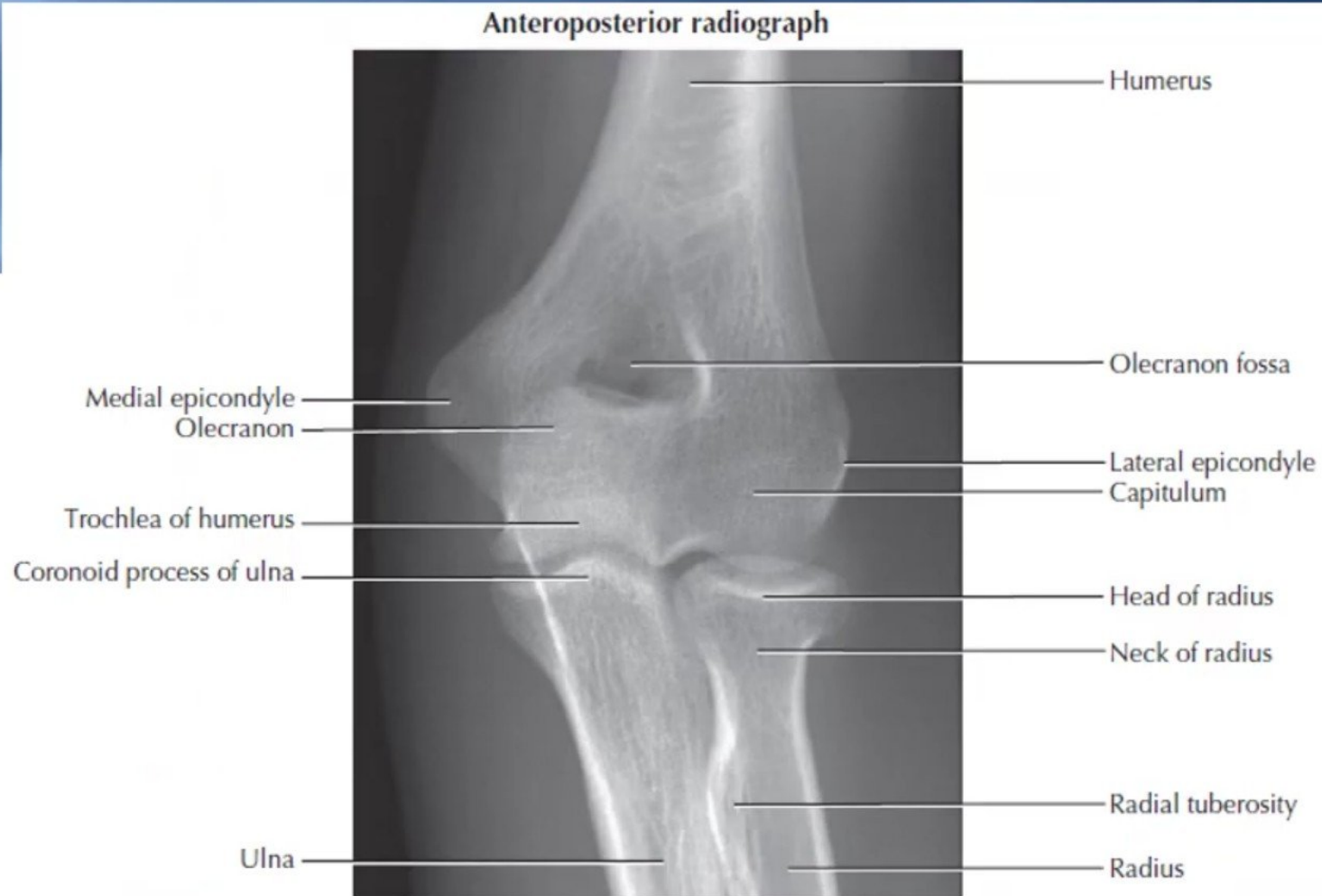


Normal X-ray Radiographic Appearances

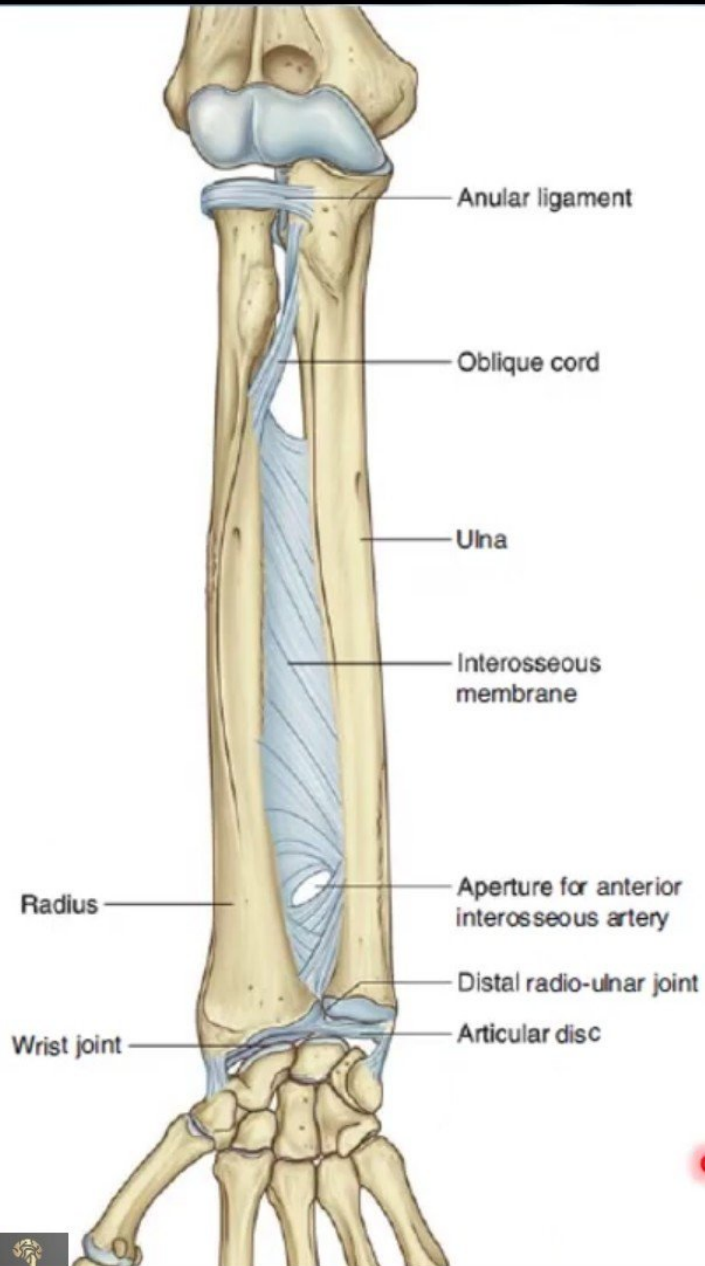
Anteroposterior radiograph of the elbow region in the adult.



In extension: posterior view

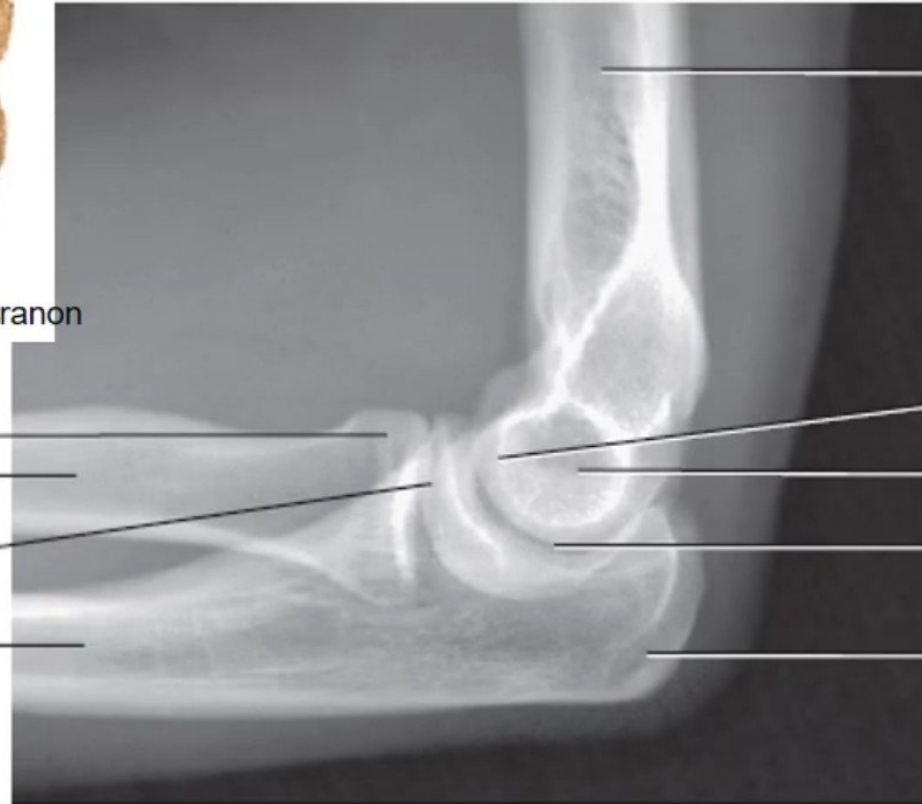
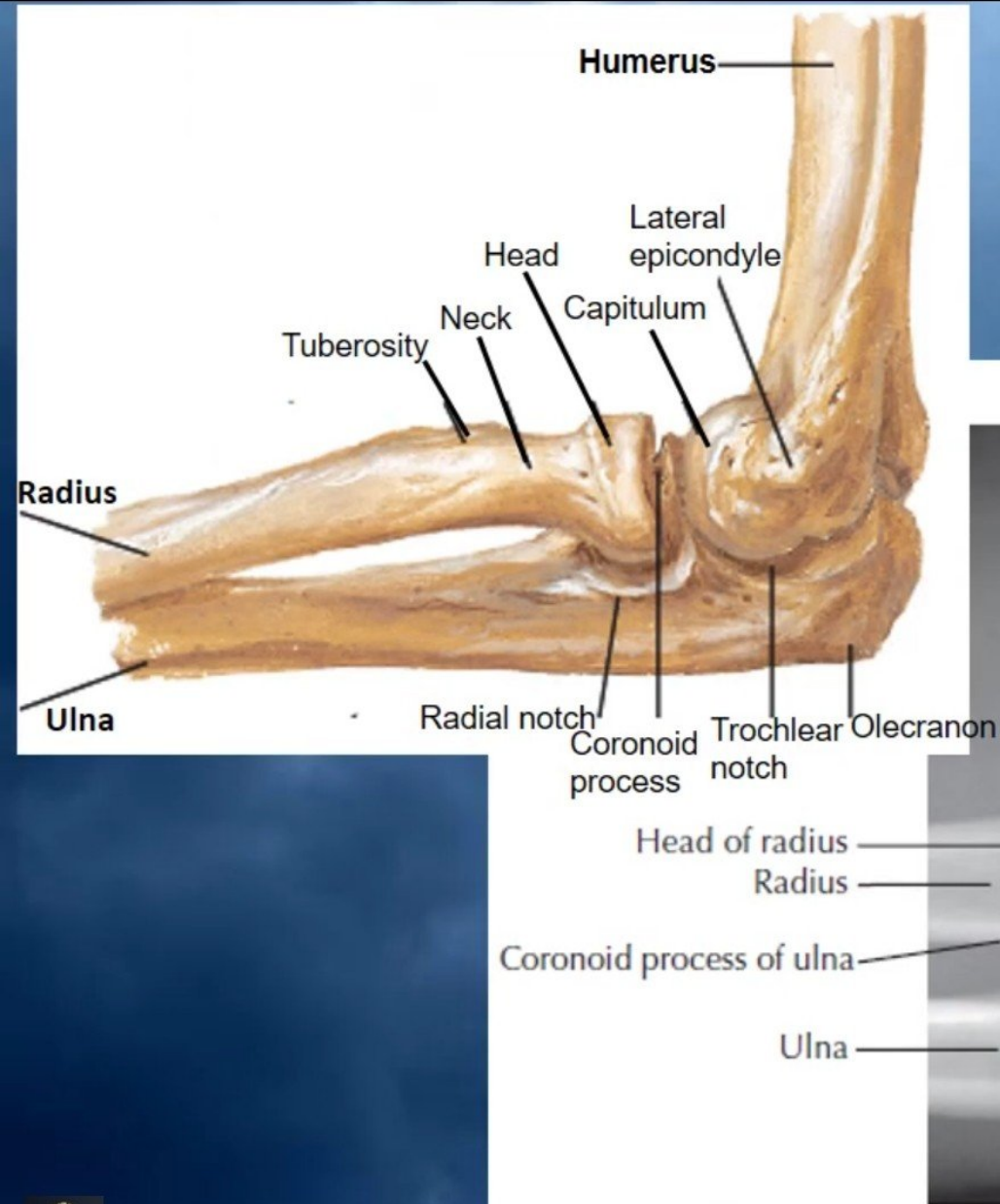


Normal X-ray Radiographic Appearances



Normal X-ray Radiographic Appearances

Lateral radiograph



Humerus

Capitulum

Lateral epicondyle

Trochlear notch

Olecranon

Fractures of radius & ulna



- Because the radius & ulna are firmly bound by the *interosseous membrane*, a fracture of one bone is commonly associated with *dislocation of the nearest joint*.

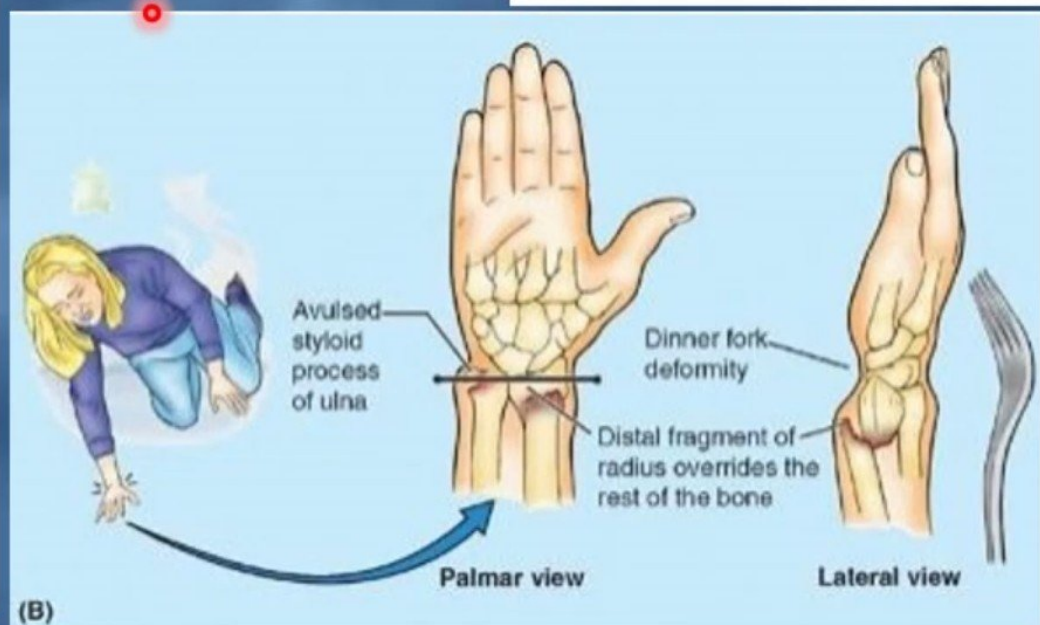
- Colle's Fracture** (fracture of the distal end of radius) is the most common fracture of the forearm.

- It is *more common in women after middle age because of osteoporosis*.

- It causes *dinner fork deformity*.

- It results from *forced dorsiflexion of the hand as a result to ease a fall by outstretching the upper limb*.

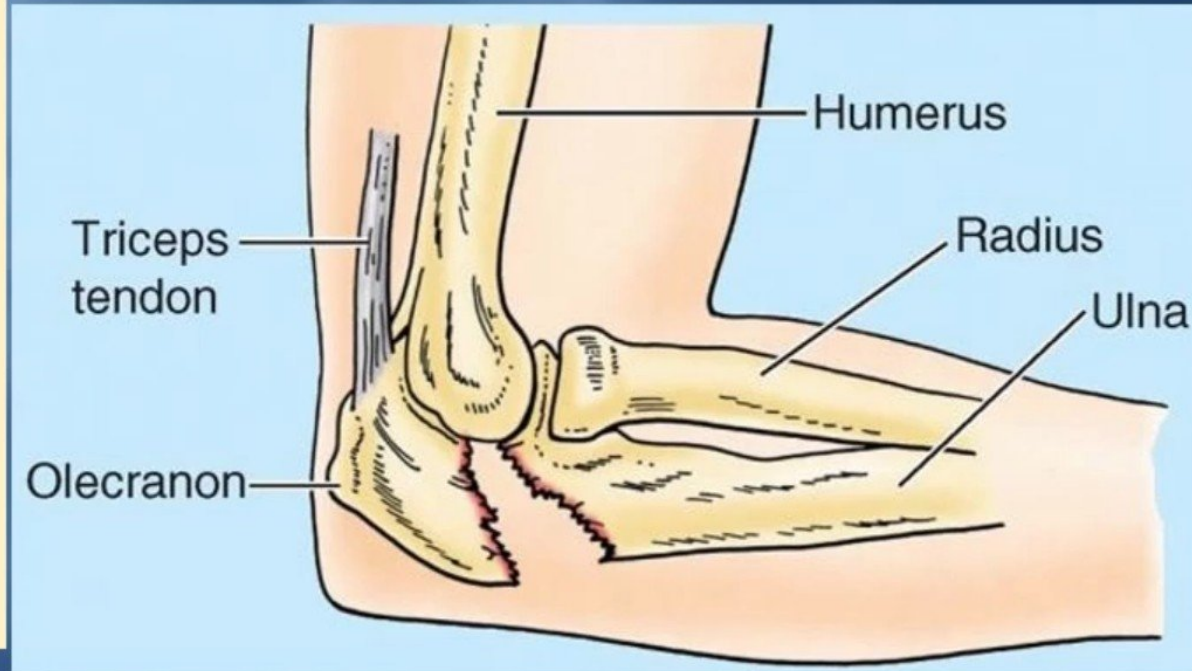
- The typical history of the fracture includes slipping. Because of the rich blood supply to the distal end of the radius, bony union is usually good.



(B)

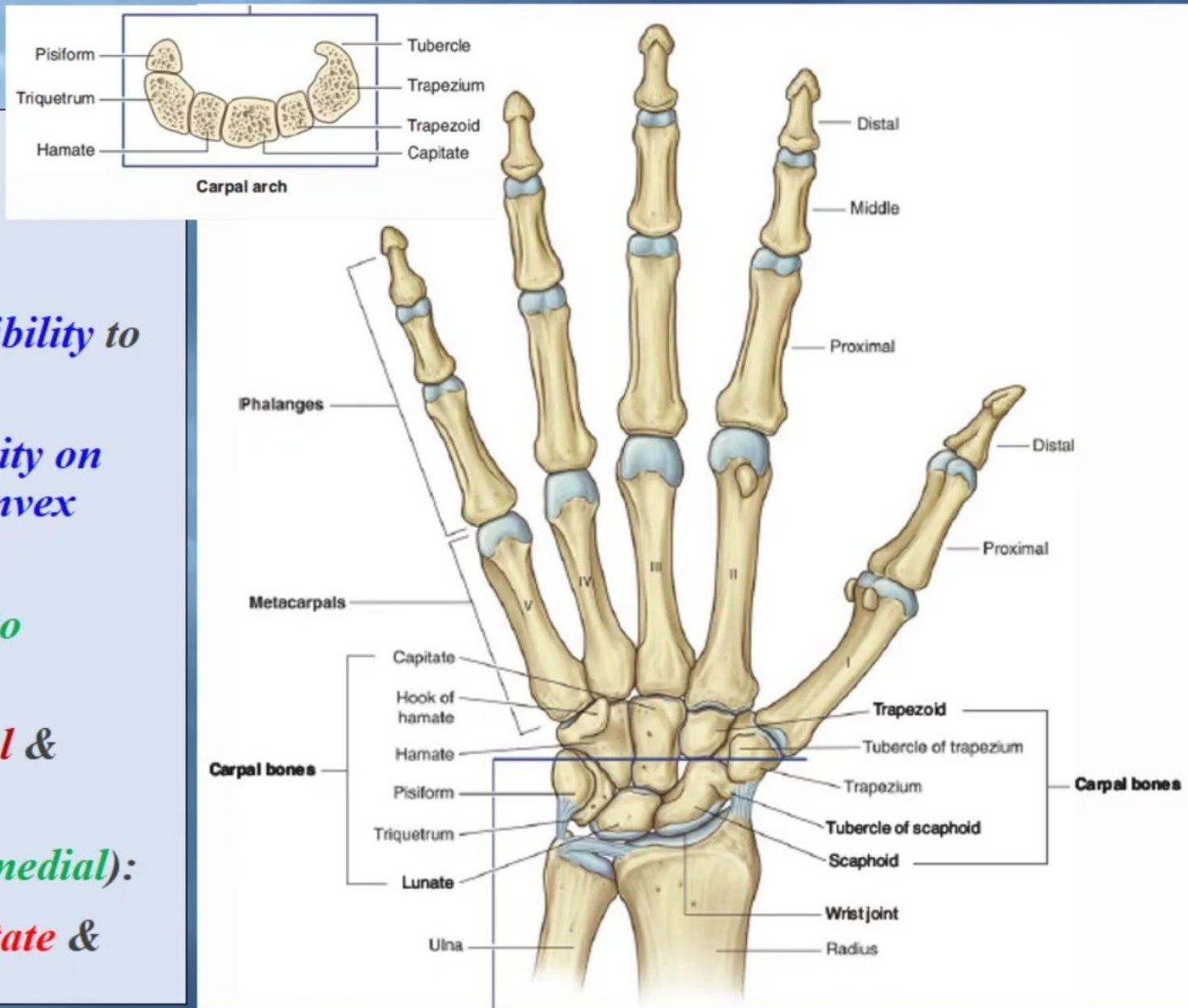
fractured elbow

- **Fracture of the olecranon**, called a “**fractured elbow**” by laypersons.
- It is common because the olecranon is **subcutaneous** and **protrusive**.
- The typical mechanism of injury is a **fall on the elbow** combined with **sudden powerful contraction** of the **triceps brachii**.
- The **fractured olecranon** is pulled away by **the active** and **tonic contraction** of the triceps.
- Healing occurs **slowly**.



WRIST (CARPUS)

- Composed of **Eight Carpal** (short bones) arranged in two irregular rows, Four each.
- These Small bones give *flexibility* to the wrist.
- The carpus presents *Concavity* on their Anterior surface & *Convex* from side to side Posteriorly.
- Proximal row** (from lateral to medial):
- Scaphoid, Lunate, Triquetral & Pisiform bones.**
- Distal row** (from lateral to medial):
- Trapezium, Trapezoid, Capitate & Hamate.**

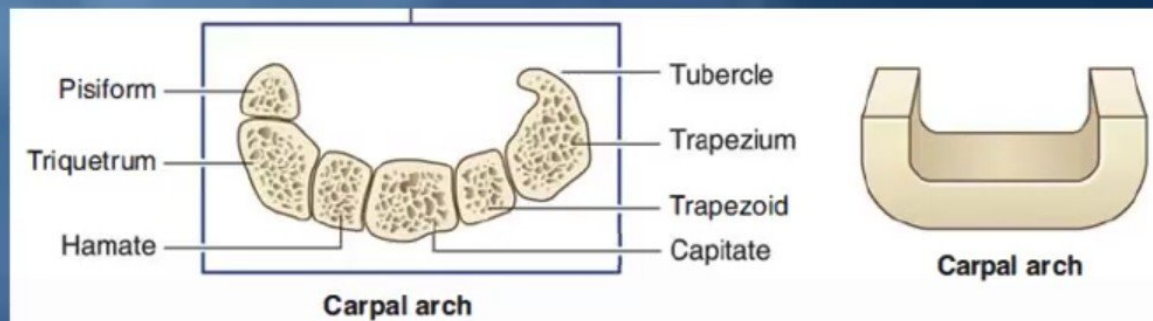
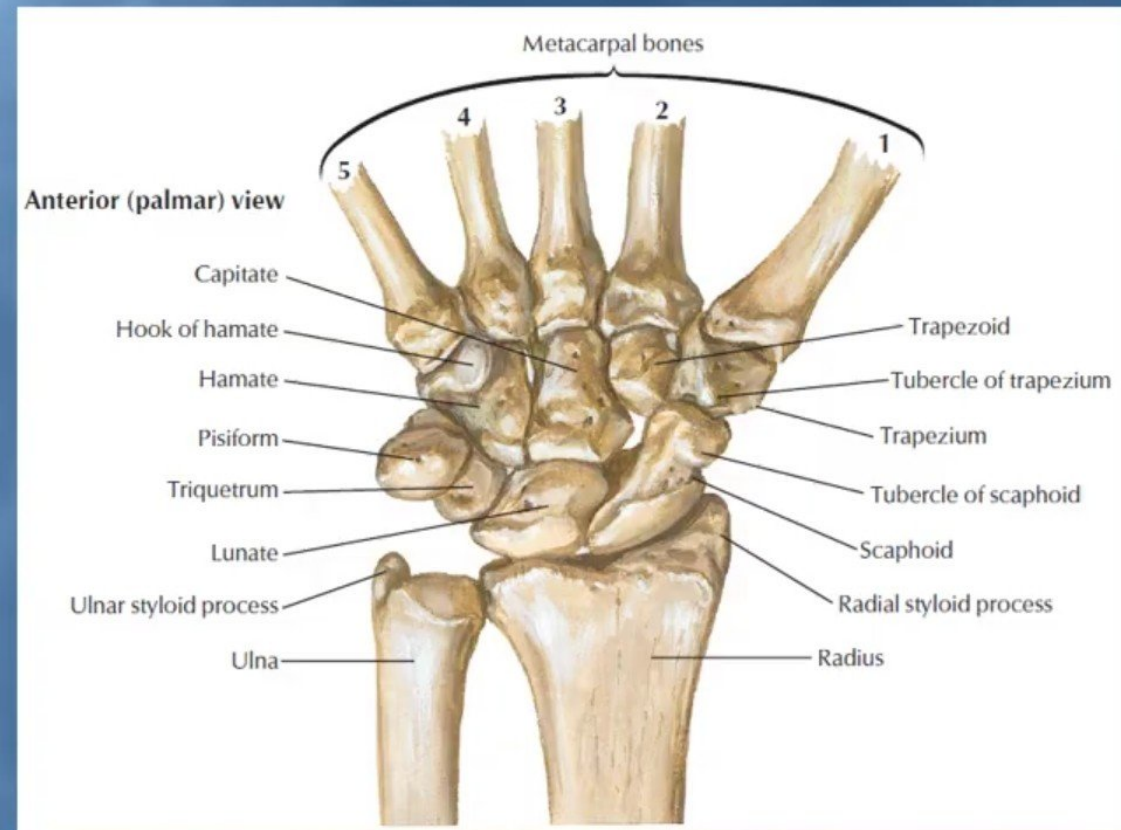


Carpal Bones

- **S**he – **S**caphoid.
- **L**ooks – **L**unate.
- **T**oo – **T**riquetral.
- **P**retty – **P**isiform.
- **T**ry – **T**rapezium.
- **T**o – **T**rapezoid.
- **C**atch – **C**apitate.
- **H**er – **H**ammate.

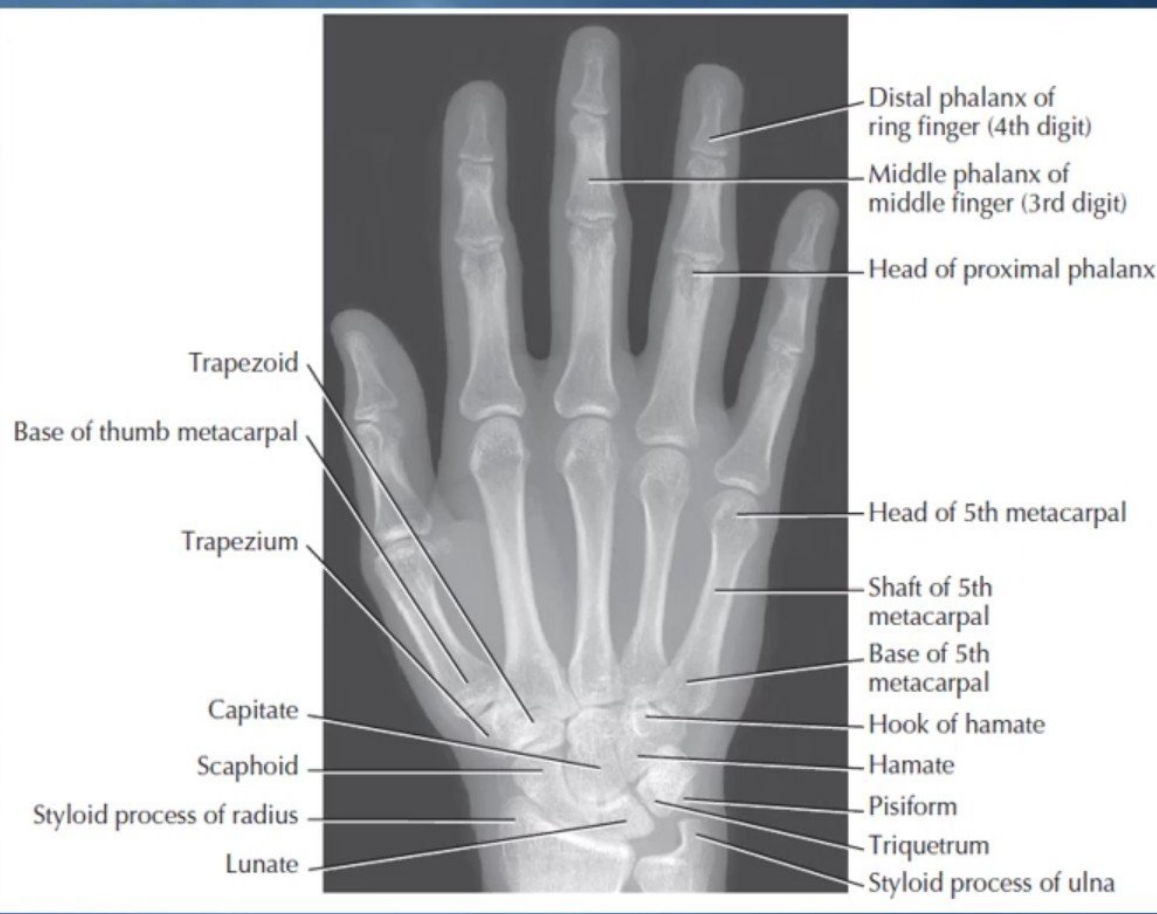
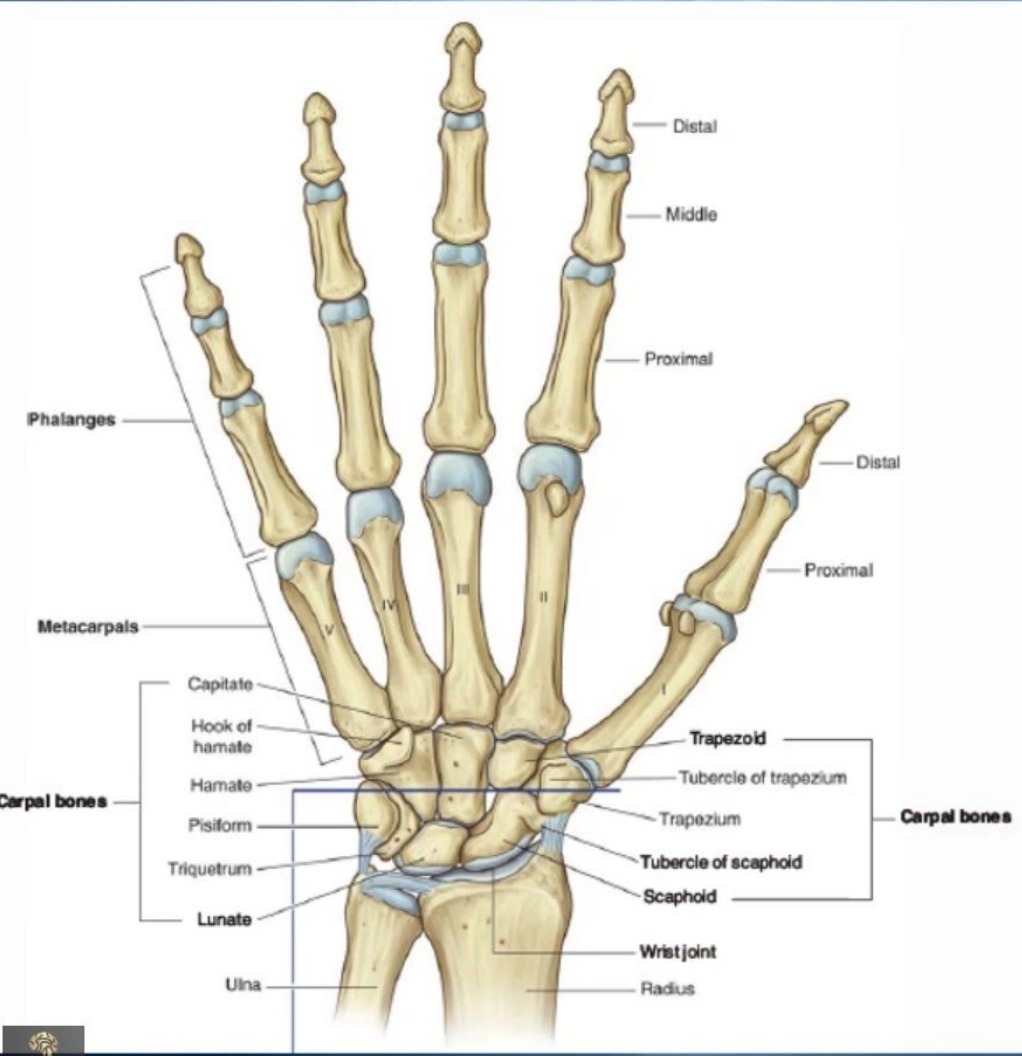
From lateral
to medial
Proximal
row.

From
lateral to
medial
Distal row.



Normal X-ray Radiographic Appearances

Posteroanterior radiograph of an adult wrist and hand



Scaphoid fracture

It is the **most commonly fractured** carpal bone and it is the most common injury of the wrist.

It is the result of a **fall into the palm** when the hand is abducted.

Pain occurs **along the lateral side of the wrist** especially during **dorsiflexion** and **abduction** of the hand.

Union of the bone may take **several months** because of **poor blood supply** to the proximal part of the scaphoid.



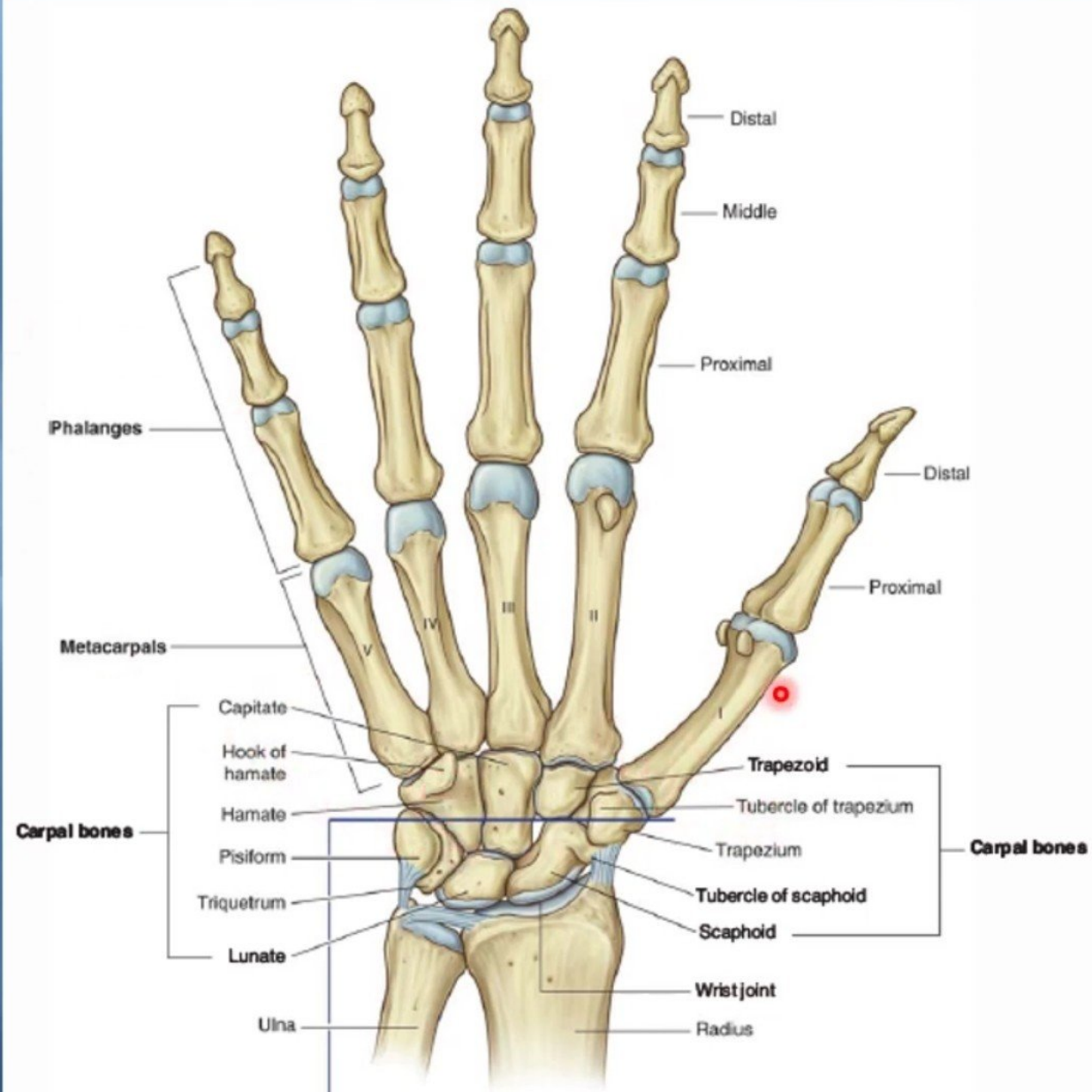
Important notes

- Scaphoid:** is the commonly fractured = avascular necrosis
- Lunate:** most commonly dislocated = anteriorly .
- Triquetrum:** pisiform lie anterior to it .
- Pisiform:** sesamoid bone= flexor carpi ulnaris / Ossify last.
- Trapezium:** groove for flexor carpi radialis ant./ radial artery post.
- Capitate:** largest ! Ossify first
- Hamate:** hook deep branch of ulnar nerve

* All ossify in cartilage by age of 7 years

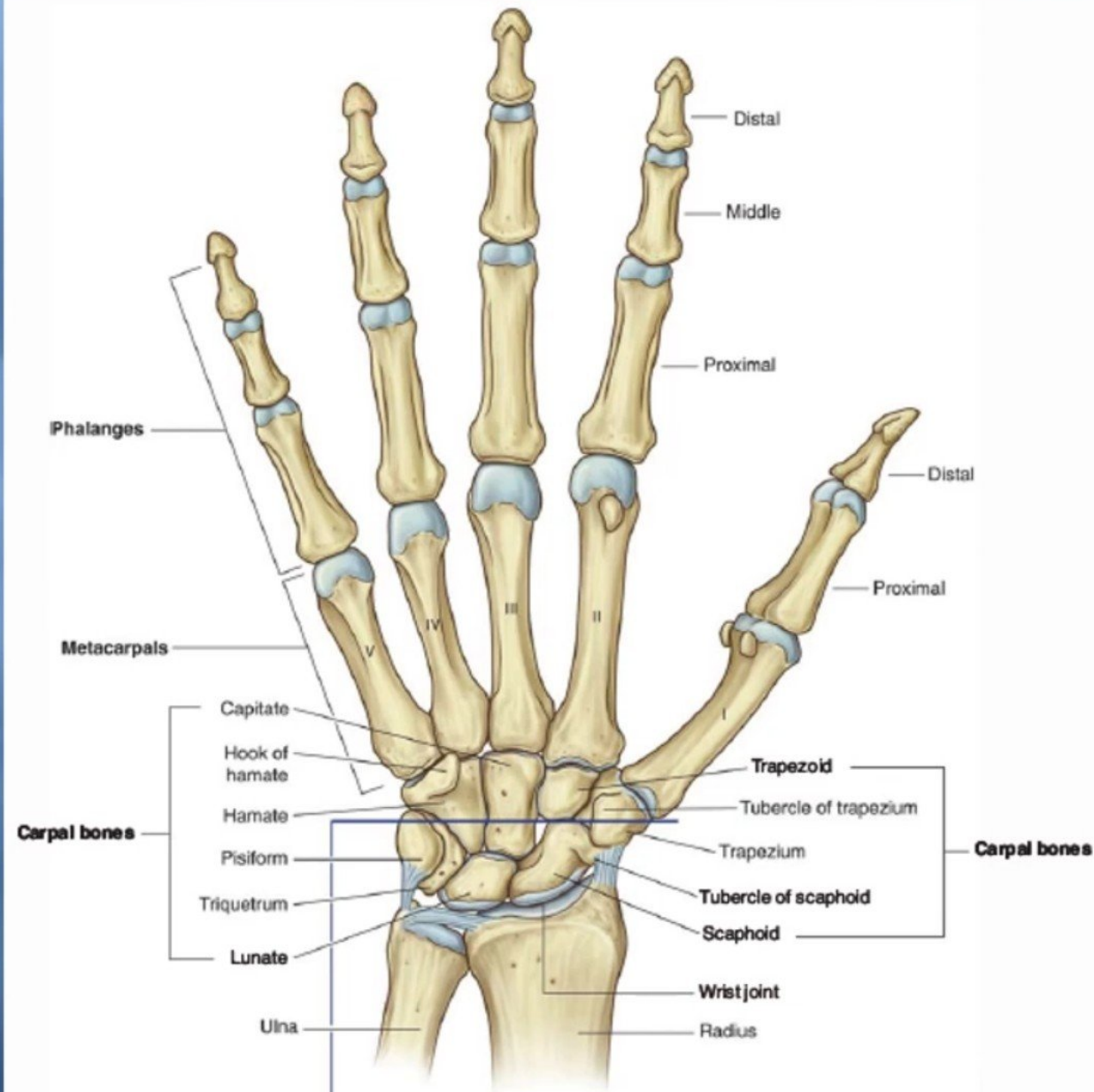
Metacarpus

- It is the skeleton of the hand between the **carpus** and **phalanges**.
- It is composed of Five Metacarpal bones, each has a **Base**, **Shaft**, and a **Head**.
- They are **numbered 1-5** from the thumb.
- The **distal ends (Heads)** articulate with the **proximal phalanges** to form the **Knuckles** of the fist.
- The **Bases** of the metacarpals articulate **with the carpal bones**.
- The **1st metacarpal** is the **shortest and most mobile**.



Digits Phalanges

- Each digit has Three Phalanges
- Except the Thumb which has only Two
- Each phalanx has a Base Proximally, a Head distally and a Body between the base and the head.
- The proximal phalanx is the largest.
- The middle ones are intermediate in size.
- The distal ones are the smallest, its distal ends are flattened and expanded distally to form the nail beds.



Thanks